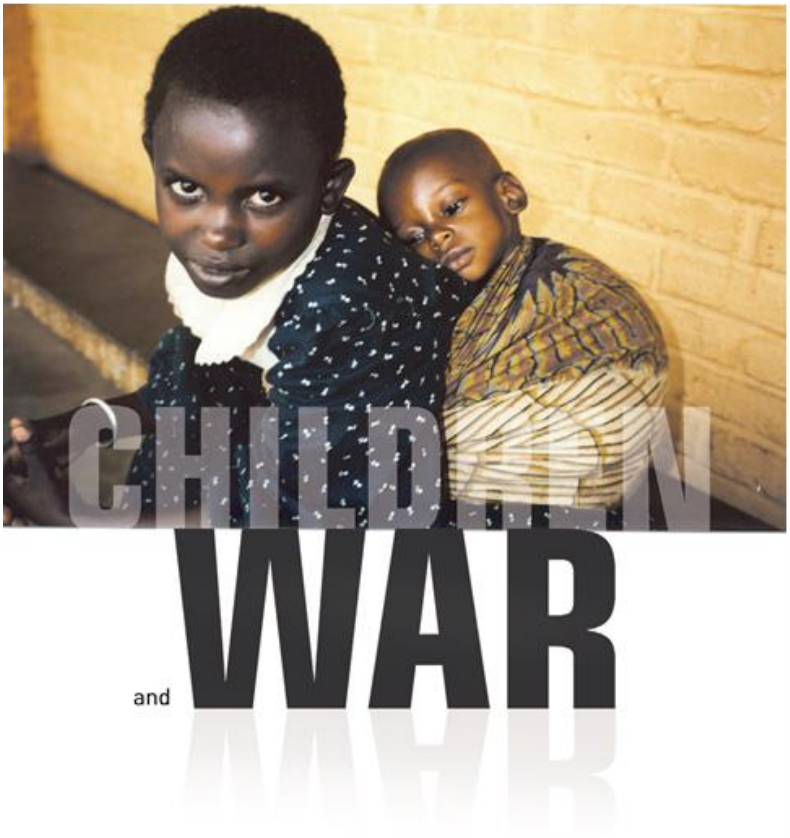
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2018

Teaching Recovery Techniques For Ages 8+



(Used for ZOOM 29&30/10/20)

TEACHING RECOVERY TECHNIQUES

CHILDREN AND WAR

8 TO 18 YEAR OLDS

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Part 1: Information for Facilitators

Introduction

Children are increasingly the victims of war and war-like events. Bombing and shelling, killings, and terrorist acts affect hundreds of thousands of children each year. Many children have fled these situations, and experience further danger as part of fleeing. As the number of affected children has risen over the years, professionals who work with children in war have sought to find new ways to help them. It is now recognised that early help for children in how to cope with the stresses of war and being a refugee can be beneficial and may prevent later problems from developing. When whole countries and communities are affected, it is often a priority to provide assistance for large numbers of children as quickly as possible.

This manual will help you do just that. It is the result of years of direct experience of working with child survivors of war and disaster, their teachers and carers, in diverse cultures and across many faiths. It is designed to help you teach children in a step by step practical way skills and techniques which are helpful in coping with the psychological effects of disastrous events. It is intended for use by teachers, youth workers, pedagogues, psychologists, counsellors, community leaders or other childcare professionals, after some preliminary training. This is not a treatment manual but is designed to prevent the need for later treatment: children who have learned and practised the techniques contained here will be less likely to need specialist treatment in the future. Nevertheless, some severely affected children will continue to need further help.

Normal reactions to war and disaster

When teachers, doctors, and parents of children living through disasters and wars in countries like Rwanda, Bosnia, Kosovo, China, Iran, Syria, Ukraine and Nepal were asked how their children had been affected they reported a wide variety of changes. Some children and adolescents had coped very well: they showed incredible resilience. And yet when the adults were asked about the children, they reported a wide variety of changes. Children were nervous, jumpy, and fearful. They were scared of loud noises, and sometimes too frightened to go outside. Some children seemed withdrawn and depressed, crying easily and getting upset over minor things. They didn't want to play and lost interest in the things they used to do. Some children became very worried and anxious. Others became more irritable and were quick to anger, or had temper tantrums. They were restless and had problems concentrating at school. Some had problems sleeping, waking up in the night crying and said that they had nightmares about what they had experienced.

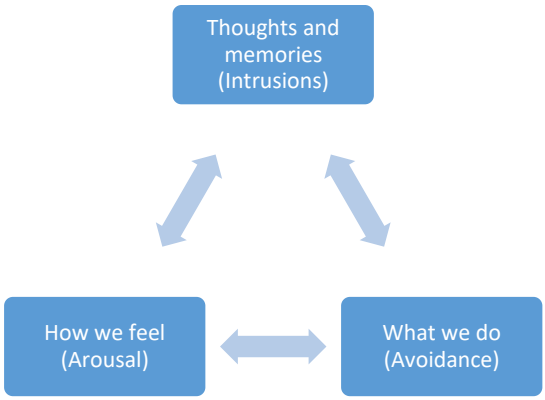
When we asked the children themselves how they were feeling, many reported problems with awful memories of the things they had witnessed or been caught up in. The memories came back to their mind uncontrollably and vividly. For some, it was as if they were living

the experience all over again. These are called flashbacks. Many feared they were losing their mind or going crazy. Because it was so upsetting to think about what they had seen, many tried to push it out of their minds, and didn't want to talk about it or to go to places that reminded them of it. Most children also said that they felt much more nervous and jumpy than before the war events. Some felt that they were losing control and didn't know what to do.

We now know from research following war and disasters around the world that it is common for children to experience these sorts of stress reactions after exposure to traumatic events. They are often called Post Traumatic Stress reactions. Historically, they were grouped as three sets of reactions:

- Intrusive memories: pictures, thoughts, smells and sounds that come into your mind without you wanting them
- Avoidance: trying to push memories away by trying not to think, talk or be near reminders of what has happened
- Physiological arousal: being on edge all the time

This type of stress comes about because the memory of the event is so strong that it just bursts into mind whether you want it or not. The natural response is to try to push the memory away, and to avoid all reminders of it, because it is so distressing. Unfortunately, while this might provide some immediate relief, in the long term it makes the problem worse. It stops the memory from being fully dealt with. The memory continues to return, often in the form of intrusive images or as nightmares. A vicious cycle develops, with more intrusive recollections leading to greater avoidance, and vice versa. At the same time the child is likely to become more fearful and anxious.



When these problems persist, the child's home and school life will be affected. If the child is re-exposed to another traumatic event or to reminders, the problems are likely to get worse. So, in this manual we have concentrated on children's stress reactions. The Sessions focus on each of the three groups of Traumatic Stress symptoms and teach children ways of dealing with them that have been proven to be helpful –i.e. evidence-based. The aim of the Sessions is to prevent these symptoms from developing into serious problems in as many children as possible and to help those children who might recover spontaneously to do so more quickly.

How to use the manual

The manual is divided into two parts. The first provides information on the background of the manual and its use. The second part provides detailed Sessions intended as a guide for you to lead five 90-minute groups with up to 15 children. The first two Sessions deal with intrusive thoughts and feelings: problems such as upsetting memories, nightmares, and flashbacks. The third Session deals with children's arousal: their difficulties in relaxing, concentrating, and sleeping, for example. The fourth and fifth Sessions deal with avoidance: children's fears, and difficulties in facing up to reminders of the war events. An additional Session contains guidelines for running a parallel parents group. A final section contains information and interventions to help children who have been bereaved. Appendix I contains alternative scripts for two relaxation exercises as well as template outlines which can be reproduced for use with the children if required. Appendix II contains alternative instructions for use when it is not possible to provide materials such as pencils and crayons.

Each Section starts with information for the Group Leader on the particular stress symptoms to be covered as well as the ideas behind the techniques the children will learn. These are introduced to the children as Tools and each technique is given a catchy name to help the children engage with the exercises and remember them. The Tools covered in each Session are listed along with any materials required. Each Session then continues with a set of practical instructions and activities to be carried out with the group, as well as tasks for children to carry out before the next meeting. These have been written in the form of a script, words which can be read out by the Group Leader. Within the wording are two types of message to the Leader which are not to be read out ……

→ *This indicates an instruction for the Group Leaders. Each instruction begins with a highlighted verb for ease of use*

Note *Provides information for the Group Leaders or a rationale about what follows*

Running the groups

Group numbers: These are best run with fewer than 15 children, but larger groups might be necessary under some circumstances. However, larger groups will extend the time each Session takes, if all children are to participate fully. The groups are intended for all children who have been caught up in the war events, not only for those who are clearly suffering psychologically. So, all children in a particular school or war-affected community could be asked to participate in the groups. Similarly, because the aim is prevention of trauma the groups should be held as quickly as practically possible after the traumatic event. Practically

this means at least 4-5 weeks after the disaster. This also allow for normal reactions to subside.

The manual does not contain individual treatment protocols. Rather, these groups are intended as secondary prevention measures. That is, the skills that children learn during the groups can be helpful for them to cope with current difficulties, but are also useful in preparing them for possible future problems.

Group composition: Although these activities may be used with a fairly wide age range of children, the children in a particular group should be of roughly the same age or maturity level. Groups would normally be mixed, with boys and girls together in a group, unless there is some other reason to carry out single sex groups. The same group of children should continue through the five sessions together. Parent's permission for children to attend should be obtained, but all children should be strongly encouraged to attend. The activities demand a certain level of cognitive maturity, so these groups are not suitable for children with learning difficulties. Similarly, some of the activities here may not be suitable for children who are known to have been severely psychologically affected by the disaster, so consideration should be given as to whether individual treatment would be more appropriate for such children.

Group timings: Each group session has been designed to be covered within about 90 minutes although it will be important to allow the children short comfort breaks and, if possible, provide them with a drink and snack during one of these. Approximate timings are provided for the tasks and techniques included in each Session. This should make it easier for group leaders to plan when to take breaks, based on the needs of their children. If group leaders decide to spread the content over more than the 5 Sessions this will not be difficult as each Session contains several separate parts. There are logical places to do this within each section. However, any new Session should begin with Remember to begin with a "warm up" and end with a Closing Game.

Group leaders: We recommend having two co-leaders to run each group because of the pressure involved in responding to the needs of war or disaster-affected children. The same two leaders should jointly run each of the five weekly Sessions with the same group of children to ensure that continuity and trust is built up. The tasks for each Session should be divided such that one leader speaks and runs the exercises while the other monitors the children's reactions to check that they seem to be understanding and are not getting upset. It can also happen that a group leader loses the thread, dries up, gets distracted or even gets upset. And it may be that they have also been affected by the same war. That is when the co-leader can step in and help the session proceed smoothly.

Group style: The style of the groups is important. The sessions are made up of a mixture of information giving and interactive, practical group activities. The groups are not intended to be like those found in some traditional classroom teaching, nor are they treatment groups where there can be a lot of emotion expressed. Instead, they are intended to be preventative, interactive, encouraging self-help and giving mutual support. The role of the group co-leaders is crucial in setting the pace and tone of the group. Co-leaders can act as models for children in showing respect and understanding of others, listening attentively, demonstrating empathy, active coping and initiating a sense of optimism while acknowledging children's reactions (problems), and in using humour where appropriate. Leaders should be prepared to hear children's war stories: children need to have the experience of talking about what has happened to them without having others pull away from or avoid them or become emotionally overwhelmed by what they have to say.

Younger children: The TRT has been adapted to meet the developmental needs of younger children. A separate manual is available, in which content and exercises do not require the children to be able to read or write.

Making adaptations to suit local conditions: The TRT has been written to be sensitive to culture. However, we encourage trainers to use their creativity and judgement and to be flexible with those techniques or suggestions that they find run counter to their culture. We would also encourage Leaders to use metaphors and images that fit in with the children's experiences and local conditions. With this in mind, Session 1 encourages Group Leaders to listen to the language young children use to describe their traumatic symptoms and where appropriate to use the words that are common within the group.

In Appendix I, we have provided some alternative versions for the breathing and relaxation exercises. These practices are more common in some communities than others. If children are already familiar with appropriate local techniques, Group Leaders may choose to adapt these for use in the Groups.

Although the Group Sessions allow for some flexibility it is important that Group Leaders stick to the wording when reading out the Little Boy's Story. This has been developed to reflect the experiences and emotions children have described. It is referred to from time to time throughout the group sessions when it is used to explore such things as triggers and reminders.

Evaluation: assessment and rating scales

Through studies from different countries and continents, the manual has proven effective in reducing posttraumatic stress reactions and other mental consequences of disasters. For a review of these results and more information on the background of the Children and War Foundation you can download an article by Yule, Dyregrov, Raundalen and Smith (2013) free from the European Journal of Psychotraumatology website (http://www.tandfonline.com/doi/full/10.3402/ejpt.v4i0.18424).

We are committed to evaluating the use of the manual. To this end, we have developed a package of self-completed questionnaires that measure distress in children. We strongly recommend that these screening questionnaires are completed by all children before they start the programme and then again afterwards. We recommend that screening questionnaires are used before the intervention, around a week after and again three months after the intervention. This will help guide local personnel in how the programme is working and help them make appropriate adjustments. We would very much welcome being sent copies of the pre- and post- results, so we too can adjust the manuals in the light of experience.

Simple, self-completed rating scales are available from:
https://www.childrenandwar.org/projectsresources/measures/

Safeguarding

This manual is written for a variety of adults who are available to help children after a war or disaster. Those responsible for running the intervention programme have a duty to screen the group leaders and safeguard the children.

We have developed these materials to reach out to large numbers of children. We believe that the techniques described can help children deal with all sorts of distress and can therefore be thought of as psycho-educational training helpful to all. We recognise that not all children are badly affected by exposure to disasters and war, although a high proportion may be. As with any public health approach, one is aiming an intervention to the total population to reduce the distress overall. There is a small risk that some children may be affected by the intervention itself. If some children disclose very frightening material in their groups, less traumatised children may be vicariously traumatised. Group leaders need to be aware of this possibility and attempt to control the level of disclosure.

For this reason, some argue that all children should be screened – preferably using a battery of tests such as we have developed, rather than simply by guesswork or impression – and the groups should then consist of only the most seriously affected. In part, a decision like this is taken on the basis of what resources are available, but we would argue that all children should be given some sort of help. Eventually, we would hope that evaluative data would help guide the decision making so that the greatest benefit can be delivered to the greatest number in the most effective and efficient way.

Part 2: Group Sessions

Session 1: INTRUSIVE MEMORIES

After a war, awful memories can come at any time of the day, out of the blue; or in response to reminders. We all know that thinking back over bad times can bring back feelings of sadness or distress. Usually these memories and feelings are not as strong as those felt at the time of the event. But sometimes images and the feelings that were evoked by the events can be just as powerful as they were at the time they happened. Some memories feel as if the whole event is happening for real, all over again. We call these **Intrusive Memories** because they force themselves, or intrude, into people's minds**.** They may come at night-time, in the form of nightmares or bad dreams. Because these memories are so vivid and frightening, they can be overwhelming, and many children worry that they are going crazy or that they won't be able to control themselves. In this session, you are aiming to show children that these reactions are normal and that they are not going crazy. You are also going to give children ways of getting control over their memories. They will not be able to forget the awful things that they have witnessed or experienced, but they can learn to remember without becoming overwhelmed with emotion.



Safe Place

Materials needed:

- ✓ Soft ball/bean bag
- ✓ Flip chart and marker
- ✓ Paper
- ✓ Crayons/pencils

Note

The first session sets the tone for the way the rest of the groups will run. They will begin and end in a similar way with everyone, including the group leaders, in a large circle. The first meeting is slightly different from the others as it involves everyone being introduced to everyone else.

How the introductions are done may vary slightly according to the age of the children and the degree of familiarity they have with each other before the group starts. For example, for groups not familiar with each other or at the younger age levels, suggest playing an unthreatening group game to start the introductions, using a bean bag or soft ball that each child throws to the next person. When they catch the ball, each person starts by saying his/her name and then add their favourite colour or food or other neutral concept - or name one or two things about themselves or something they are good at. A second round might be needed for this first group warm-up – so maybe "my favourite [something]" first time and "I like to [do something]" or "I am good at [something]" for the second round. For older children an alternative approach is for children to pair up with their neighbour for a brief chat to ask for and exchange information before each introduces their partner to the group, giving their partner's name and a couple of details about them.

Hello everybody. It's good to meet you all. Now, you may all know each other but if you don't and because we've only just met, let's introduce ourselves.

Opening Option 1. Younger children (or as you think appropriate)

What I want you to do, so that we can get to know each other, is to sit in a large circle and to throw this ball to each other. When you catch the ball say your name and your favourite colour/favourite food/one or two things about yourself. I'll start. My name is… and my favourite food/colour is…

→***First trainer starts*** *the introductions*

Opening Option 2. Adolescents

So that we can get to know each other, instead of introducing ourselves let's do it a little differently and introduce a partner, saying their name and a few things about them. Then your partner does the same for you. We (trainers) will show you first.

This (point to second trainer) is -----. He/she comes from -----. He/she has… (e.g. a dog) and likes… (e.g. to go running at weekends). Now my partner will introduce me.

→***Second trainer introduces*** *first trainer*

Now you get into pairs with the person next to you (starting from here). Take a few minutes to have a chat – make sure you know your partner's name and something you can tell the group about them. Keep it simple and short – and make sure you each have a chance to ask things of the other. Don't worry – you don't have to remember everything, and it is ok to help your partner if they don't get it quite right.

→***Say,*** *for both Opening Options, after the first round of introductions*

…. Phew, that's a lot of names. Let's go around one more time, say your name again and one (or two) more things about yourself/your partner.

Let's see if I can remember all the names now.

Note

Letting the children know why you are meeting and that this is the first of five meetings. It is important to be direct about the fact that you will be discussing the war and how it affects children. Say that this might be difficult and upsetting at times, but also engender a sense of hope and optimism in coping with difficult feelings.

Well done everybody! We're meeting today because you have all lived through some terrible things during the war. We know that for grown-ups and young people, after they have seen or experienced terrible things, it can be hard to live with those memories. Wars affect everyone, but different people are affected in different ways, and there's no right or wrong way to feel. It can be difficult to talk about what you've been through but in these meetings, we are going to show you some tricks and things that you can do to make it easier to deal with the memory of what you have experienced. We call these things TOOLS and during the meetings you will get different tools that you can put in a mental TOOLBOX. The idea is that we can use these - and help each other to cope better. The tricks we are going to learn do not take away any memories, just make the intrusive ones easier to control.

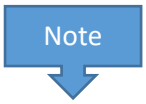
Note

Setting ground rules for confidentiality and respect. This will help the youngsters feel safe about expressing difficult feelings and

experiences. It is important that these rules are generated by the group, not imposed by the group leaders. Help the children to generate the following and write them up on a flip chart/board.

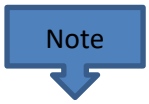
Because we'll be talking about hard things sometimes, we need some rules to make us feel comfortable - remember that we're on each other's side and we'll be working together as a team. Let's write them up here... Who's got an idea?

- ✓ what is said here stays here (confidentiality)
- ✓ respect for each other
- ✓ no one has to talk but everyone should listen
- ✓ go at your own pace
- ✓ talk only about yourself not about others
- ✓ listen until someone has finished
- ✓ no putting down of others
- ✓ and come to all meetings



Starting to face what has happened and learning about reactions to war stress. This section helps the youngsters define their own problems, normalises their reactions and helps the group come together as they share stories.

Now that we've all got to know one another and we've set some rules, we can start to talk about our own bad memories, and learn some ways to get better control of them. But before we do that, I want to tell you about a young boy and what happened to him. He's not anybody you know, but some of the things that happened to him might have happened to you as well.



Through the story that follows the children are gradually getting used to hearing about and later talking about very difficult things. The story has been written to include examples of all 3 types of trauma responses – uncontrollable memories; avoiding memories or reminders; feeling very stressed in body and mind. It has also been written to include the sights, sounds and events children experience during war as these often become triggers for stress responses.

→***Read aloud*** *and change the name to make it culturally appropriate*

Michael's Story

Michael was about your age when the war started. He lived at home with his mother and three sisters, but his elder brother and his father were in the army. One day as he was walking through the main street on his way to school, a big truck came very fast down the street and stopped quite near him. Many soldiers got out shouting and they began to shoot their guns loudly into the air. Four soldiers ran over to two old men who were sitting near the side of the road. The soldiers dragged the men shouting over to the truck. Michael thought that they would make the old men get into the truck. Instead, as he watched, they made the men stand up and two of the soldiers shot the men in the stomach several times. The men fell over onto the ground and Michael saw lots of blood and heard lots of screaming. The truck drove off fast. Michael ran home crying and learned later that the men had died.

How do you think Michael felt after this experience?

→ ***Listen*** *to children's reactions as a step to normalising them. Then return to the story, saying:*

This is exactly what happened to Michael.

Michael got very frightened over the next few days. He didn't want to be far from his mother, and couldn't bear to go back to the place on the street where the men had been shot, so he stopped going to school. He didn't like to think about what he had witnessed because it made him nervous and afraid and he started to cry. He didn't want to talk about it to anyone because it was too upsetting. Still, he couldn't get rid of the memory of what had happened. Whenever he heard the sound of gunshots, his heart leapt and he suddenly saw the picture of the men being shot in his mind. It was just like he was back there again, and he remembered particularly the sight of all the blood coming out of the men's stomachs, and it was as if he could smell the guns all over again. He

got terribly frightened when this happened and felt as if he was going mad because there was nothing he could do to stop the pictures coming back to his mind. He started to have bad dreams where he would see the killing all over again, and again it was the blood in his dreams that he was most upset by. This lasted for a few weeks. Michael could not go near the spot where it had happened, and he hated to think or talk about it because it was too frightening. The pictures kept coming back during the day and at night-time. He got more nervous and jumpy, and would jump out of his skin when he heard gunshots, or if he heard a car or truck coming, or other loud noises.

How did Michael react to what happened and how did he feel? Let's make a list of them on this chart/board.

→ ***Begin listing*** *these Symptoms and behaviours on a flip chart*

The sorts of things you've noticed about Michael in the story are quite normal after the sorts of experiences he had. What other events do you think can cause these sorts of thoughts, feelings and reactions? Let's talk about this. Let's make a list of these events. Who wants to start?

→***Elicit*** *examples such as floods, fires, earthquakes, traffic accidents, personal violence*

Note

Learning about what makes an event traumatic. This helps the children understand that reactions to traumatic events are common, can happen to anyone and has a name.

So, looking at this list, what sorts of things have these events got in common?

→***Elicit*** *ideas*

Events that are:

- ✓ sudden
- ✓ life threatening: can cause injury or death
- ✓ involve intense sensory experiences: sights, smells, sounds, body position, tastes
- ✓ horrible
- ✓ make people feel terrified and helpless at the time

we call TRAUMATIC events.

The sorts of things that Michael was experiencing are the sorts of things that people might experience following ANY traumatic event.

Previously most children wouldn't have experienced a traumatic event but now, because of the war, they are all too common. The reactions that Michael had after he saw the shooting can happen to anyone. In fact, most children – and adults too -have similar sorts of problems after they've seen terrible things. It's normal to have these sorts of reactions, and some people call them Stress Reactions– or even Post Traumatic Stress Reactions.

Note *Normalising the children's reactions through sharing*.

Has anyone here had problems like Michael? Any different reactions?

→***Find/add*** *the problems they come up with to Michael's symptom list*

It's very common for children and adults to have these sorts of problems after a very scary [use their word for traumatic] event. After you've seen something terrible, when things remind you of what happened, it can feel like it's happening all over again, even though you know it isn't. It's very frightening to feel like that, so children try to avoid places that bring back the bad memories and feelings, and they avoid thinking and talking about what happened, even to their parents. When this happens to you, it doesn't mean you're going crazy. In fact, it means that you're just like lots of other children who are caught up in wars. Even though you might try to push the memory away, it still comes back, maybe in your dreams, or when you hear loud noises like gunfire or other things that remind you of it.

Note *Understanding that memories and other reminders can bring bad feelings and that being able to identify these can increase a sense of control.*

What sorts of things remind you of what happened? What sets you off? Maybe it's the sound of a car backfiring, or something you see. Let's see if we can make a list up on our flipchart/blackboard.

> →***Wait*** *and… if no-one answers* ***ask****…What reminded Michael …Now what about you?*
>
> →***List*** *reminders on a flip chart/board. You will use these again.*

So, you can see there are lots of things that can remind you of what happened and make you feel upset again. It's impossible to avoid everything and because the reminders can often be unexpected and everyday things it is easy to feel that everything's out of control. And sometimes, if you find yourself feeling upset and don't know why, you might be able to find a reminder that has triggered it. Maybe that's something you can keep an eye open for to help you feel more in control.

Note

Providing hope by showing how children can take charge of their own thoughts.Introducing the idea of TOOLS for dealing with thoughts, feelings and behaviours.

So, even though you may try to push the memory away, it still comes back in the end, either when there are reminders around, or when you are asleep, or sometimes just out of the blue. Instead, what we'll do over the next few meetings is to learn some tricks and special things to do so that you are the one in charge of your memories and you can control them better. They're a bit like TOOLS to help you cope better. You won't be able to forget what happened to you, but you will be able to remember it when you decide, not when your memory decides, and it won't be so scary to remember. You'll be able to use different TOOLS from your TOOLBOX of tricks to handle different situations. You can practise them in the group and between the groups, and in the future if you need them. The important thing is to practice the skills that you learn, so that you'll be ready to handle bad memories if they come up in the future

> →***Consider*** *whether the children need a short break or stretch/move around/jump up and down before continuing*

Note

Before starting on difficult memories and images it is helpful for the children to practise creating a "Safe Place" in their imagination. This allows them use their imagination and manipulate the pictures in their mind in a helpful way in which they take control. Children should be encouraged to use this Safe Place if they feel upset during the groups.

In this first activity, we're not going to think about bad memories yet, but we'll practise using our imaginations to create positive/nice images and feelings.

Sometimes when we are upset, we might imagine a place or a time which makes us feel better, calmer, more secure. I'm going to ask you to imagine a place or scene that makes you feel like that. It could be somewhere real that you remember, maybe from a holiday; or it could be somewhere you've heard about, maybe in a story; or it could be somewhere that you invent and make up yourself

TOOL: Safe Place

→***Allow*** *enough time for the children to think of somewhere*

→***Pace*** *your instructions, speaking in a rhythmic steady way.*

→***Use*** *all the senses when creating a mental image*

Take a few deep steady breaths. Close your eyes and carry on breathing normally. Bring up a picture of your Safe Place and imagine that you are standing or sitting there. Can you see yourself there? In your imagination, take a look around. What do you see? What can you see close to you? Look at the details of it and see what it is made of. See the different colours. Imagine reaching out and touching it. How does it feel? Now take a look further away. What can you see around you? See what's in the distance. See the different colours, shapes, and shadows. This is your Safe Place and you can imagine whatever you want to be there. When you're there, you feel calm and peaceful. Imagine your bare feet on the ground. What does the ground feel like? Walk around slowly, noticing the things there. See what they look like and how they feel. What can you hear? Maybe the gentle sounds of the wind, or birds, or the sea. Can you feel the warm sun on your face? What can you smell? Maybe it's the sea air, or flowers, or your favourite food cooking? In your Safe Place, you can see the things you want, and

imagine touching and smelling them, and hearing pleasant sounds. You feel calm and happy.

Now imagine that someone special is with you in your place. [With younger groups especially, this might be a fantasy figure or a cartoon superhero.] This is someone who is there to be a good friend and to help you, someone strong and kind. They are there just to help you and they'll look after you. Imagine walking around and exploring your Safe Place slowly with them. You feel happy to be with them. This person is your helper and they're good at sorting out problems.

Just look around in your imagination once more. Have a good look. Remember that this is your Safe Place. It will always be there. You can always imagine being here when you want to feel calm, secure, and happy. Your helper will always be there whenever you want them to be. Now get ready to open your eyes and leave your Safe Place for now. You can come back when you want. As you open your eyes, you feel more calm and happy.

Well done! What was that like? Does anyone want to let us know about their Safe Place? How did it make you FEEL being there? Good.

> →***Elicit*** *from a few more children their idea of a Safe Place and* ***point out*** *the link between what they have created (in their imagination) and how it makes them feel.*

So, you can see that you can choose what to see in your mind and that can make a difference to how you feel.

And your Safe Place can be the first tool for your Toolbox. You can take it out whenever you need to, whenever you feel sad, or scared.

Ending the Session

Note

We suggest that each group is ended back in a circle with a routine so the session feels rounded off for the child and there is an atmosphere of group solidarity and support. At the end of Session 1 regular "homework" (things to try out before the next session) is introduced. Some suggestions are:

- *"Favourite" or most helpful part of the session (especially for Session 1 as only one tool has been introduced)*
- *The names of the tools learned so far (for Sessions 2-5)*
- *The favourite tool learned today (for Sessions 2-5)*
- *Let each child suggest one nice thing he/she pledges to do before the next meeting*

The actual pattern of the group endings may vary according to the local culture and traditions, but could also include:

- *A special song known to the children*
- *(if appropriate), the children "send" a handshake around the group, starting with the trainer, or do a fist bump or "high fives" around the group, again starting with the trainer*

Thank you for coming today. It was lovely to meet you all. You've all worked very hard and talked about some really difficult things. Well done. And you've also got the first tool for your Toolbox – your Safe Place – which you can take out whenever you need to, whenever you feel sad or scared. Do have a go anyway at practising your Safe Place a few times before we meet next time, so it is easy for you to use when you need to.

Now does anyone have a favourite part of today's session? Or something you found helpful? Next time we'll ask you about how you got on with practising your Safe Place. But also, could you try to give yourself one nice thing to do before next time – even something little? Has anyone got an idea? Good. And what else could anyone do? It doesn't have to be something different for everyone, just something you would enjoy. Good. And before we all go, let's do one more thing [here choose from the list above or something equivalent].

Session 2: INTRUSIVE IMAGES, WORRIES AND DREAMS

The following series of techniques are designed to give children more control over distressing intrusive images, during daytime or as nightmares. Sometimes it is not intrusive memories but intrusive thoughts and worries that keep entering a child's mind. It is often the feeling of being out of control that intrusive phenomena can give rise to which are most distressing to children. These techniques aim to counter those feelings of lack of control. They will help to re-establish the child's sense of mastery and coping. As such, the style in which these techniques are taught is important, and it should be active and hopeful without belittling or denying the seriousness of the children's difficulties. Although you will be working directly with children's traumatic images in this section, remember that this is not an individual treatment session, and so the group should not become too heavy. Rather, a sense of active coping should be developed. Not all techniques will work for all children. Everyone in the group should be encouraged to try out each technique fully and see which works best for them. It is not a failure on the child's part if a particular technique does not work for him or her, and this should be made explicit at the beginning.

The common goal of these various techniques is to develop the child's ability to deliberately bring up their intrusive image, to change the image in various ways, and to be able to switch it off. For example, the following characteristics of visual images can be changed: brightness, contrast, focus, movement, distance, detail, colour, speed, location, form, duration, dimension, clarity, posture and depth.

If children are bothered by intrusive memories we recommend that they spend 10 minutes a day to play around with the memories, calling them to their mind, introducing changes, moving them around and in many ways taking control over them. By introducing positive elements, a change of perspective, a magic helper or a happy ending they will gain control over the intrusive material and not vice versa.



Picture Changer
Sound Changer
Smell Changer

Good Feeling Button
Tapping Trick
Dream Planner
Worry Time
Doing Something Different

Materials needed:

- ✓ Soft ball
- ✓ Flipchart/board
- ✓ 3 or 4 different smells either as bottled essential oils or from items found locally e.g. a lemon, a flower, herbs, spices, cheese etc.
- ✓ Paper and Crayons/pencils

Note

Start the group (and all further groups) in a circle as in Session 1. If the names are still not known by all, then start by each individual saying their own name and something positive – e.g. that they experienced since the previous week or that they liked from the previous group. Once the names are known by all, there is no need to keep on saying them, but the circle time warm-up is still important. After a greeting the homework set the previous week is reviewed. Reviewing what the children have tried between sessions is very important as it encourages further practice and so increases skill. It also shows you are validating their efforts.

Hello everybody. Welcome back. Today we are going to look at how you can learn to deal with any unwelcome, scary memories and thoughts and images that you are experiencing.

But first let's make sure we all remember everyone's names – does anyone know everyone's names? Can you tell us? Or should we introduce ourselves again?

You all said you would try to do some nice thing between our last meeting and today – who would like to be first to tell us what they planned to do and how it went. Good. And who else…?

Now let's see how many of you managed to do the homework from last time- practicing your Safe Place? Who managed it? Were there any problems?

Remember, not everyone reacts in the same way. We will be showing you a number of tools to help you get back in control of your feelings and reactions. Some tools work for some people; some for others. Each one of you will have to try them out to see which ones work best for you.

During these exercises, you will have to remember bits about what happened. It is possible that some of you may get a bit upset. If you do, what do you now know you can do about it?

→ ***Pause*** *then congratulate or remind them.*

Yes – that's right, go to your Safe Place.

Note

Visual images are the most common intrusive re-experiencing that occur after exposure to traumatic war events. Learning to manipulate these images by deliberately bringing up a visual memory, changing size, colour, clarity, orientation and finally turning it off helps reduce the impact of the picture as well as giving a sense of mastery and control.

Next, we're going to do some exercises to show you how to change the pictures of the traumatic event that pop into your mind when you don't want them to. To do this, you need to deliberately bring up the picture so that you can see it. Remember that you're the one in charge of your memories, and you can go back to your Safe place afterwards if you want so that you can feel calm again.

TOOL: Picture Changer

i. Screen technique

Keep your eyes open. Imagine that you can see a television screen on the wall opposite you. Get a good picture of the television screen. Now, try to see on this screen the picture that you get of your traumatic event. Can you do that? Good. Do you see the image in colours or in black and white? Can you change this? If it's in colour, try to make it black and white. If it's black and white, make it coloured. See what happens. Good. Now is your image moving like in a film? If it isn't see if you can make it move. Good. Now imagine using a remote control to freeze frame the action. Now start it again. Try to rewind the image and watch it playing backwards. Can you do that? Good. Play it forward now, but slowly;

and now see if you can speed up the action. Now let's try to make the picture fuzzy and blurred like when the reception is bad.
See if you can make the picture just fade. Now imagine that you take your TV controller and press the off button. What happened?

→ ***Elicit*** *feedback but reassure as below if no changes were noted.*

Well done - Some of you may have found it a bit hard to change the picture or maybe you all did. That's fine. It's just about having a go. Different things suit different people. You might find another method better for you. Let's try another one.

→ ***Choose*** *1 or 2 of the techniques below. It is helpful for the children to have a selection to try. Different ones appeal to different youngsters*

ii. Hand and distancing technique

In front of you, can you try to bring up the image of what happened? How far away is the image from you? Can you try to move the image further away from you, and while you move it let the image become smaller and smaller? Can you imagine it moving so far away and getting so small that it disappears completely? Can you imagine it disappearing into the wall over there?

Now bring up the picture and imagine that you see it on my hand. Can you do that? Watch it get smaller and smaller as I slowly move my hand away. Is it getting smaller? Can you still see it? [Close your fist and move your hand behind your back]. Now it's gone. Let's try it again.

iii. Framing.

Now, bring up the image, and put a frame around it, like a picture frame. Now we're going to try moving it around. Move your picture in its frame further away and now turn it back to front? Good. Now turn it back again and move it frame further away from you. Is it becoming smaller? Good. Now, turn it upside down. Good. Let the picture fall out of its frame. See it smash onto the floor into lots of little pieces and scatter everywhere. It's gone. Good.

iv. Positive counter image (switching from an unpleasant to a neutral or pleasant image)

This time you're going to start with a good picture, something positive you can use to help you block out your intrusive image. So, with your eyes open bring up a nice picture. Imagine this good picture become bright and strong. Let this picture be so good and big that it blocks out the intrusive picture.

Next put the intrusive image up on the screen and imagine the good picture just in one corner. Let them change places so the bad picture or image only has a place in the corner of the good picture. Move the bad image around all corners of the screen and then close "the program" so only the good image remains.

v. Introducing an Imaginary Helper

How about getting someone or something to help you change your image? It could be a person, or an animal or a character from a book; maybe a Superhero like Batman or Spiderman, someone with super powers to help you change what's happening. See if you can choose a Helper now and think about what they would do to help protect or save you. Maybe they can change what's happening …or maybe they can make something different happen which will make things feel better

So again…bring up your picture so you can see it. Keep your eyes open …and when you want to – get your Helper to come in. See what they do. Maybe they're helping sort the picture out. Or maybe they're folding the picture up so it's all hidden.

vi. Locking away the image

Now try another one. Bring up your picture again. See if you can fold it up. Bring the sides together. Now fold them again so they're even smaller. Now think of somewhere to hide them or lock them away. You can put them in a box, lock it up and throw away the key.

vii. Switching off

And one more thing you can do to your pictures is just switch them off – Press the off button, turn the knob… get your TV controller and turn it off! What happens?

Note

Visual images are the most common kind of intrusive re-experiencing phenomena that occur after exposure to traumatic events. Still, children may report intrusive phenomena in other sensory modalities. Group activities, similar to the visual imagery techniques above, can be useful in countering the uncontrollability of other sensory intrusions. They can be applied for children that experience uncontrollable, intrusive re-experiencing of sounds (e.g. screaming, shelling, or gunfire), smells (e.g. burning flesh, gunshots), and bodily sensations (e.g. being buried or thrown around by shell impacts).

Note

Hearing things. Although it is common for sounds to remind children of situations and trigger distress (e.g. door slam- reminds of gunshot) auditory re-experiencing is rarer.

Sometimes, as well as having bad memories that are pictures, children can have bad memories that are sounds. Some children say they hear people crying or screaming in their head when there is no one really there. Some children say that these come mostly at night time as they are trying to fall asleep. These are not real screams, even though it can sometimes seem like they are, but they are memories which can come back even when you don't want them to, just like the pictures we were working with earlier. Now we're going to practise some ways of turning these sound memories off and on, just like we did with the pictures.

TOOL: Sound Changer

Note

(Auditory) Sound memories. Dimensions to change include: pitch, duration, volume, suddenness, accent, clarity, distance etc.

For this part, I want you to deliberately bring up the sound memories that disturb you. If you don't really have any, then you can practise this exercise with some other sound you remember - you should try to practise it even if you don't have these problems now, because it might become useful in the future.

First, just listen to the sound in your mind. Notice how loud it is. Is it clear or muffled? Is it intense and sharp, or vague? Is someone speaking? Is it a man or woman? What is their accent like? Now I want you to bring up the sound memory again, and this time, imagine that it is coming from a radio (or TV

without the picture, etc.). Can you hear it? Now, imagine reaching out and turning down the volume control. What happens? Turn it down further, and further, and the noise gets quieter and quieter. Now try turning the volume control up. Does the memory get louder? Turn it down again. It gets quieter and quieter. Turn it down until you can't hear it any more. And switch it off. Good. Let's try another one.

→ ***Choose*** *another quality of sound to manipulate e.g. if a child remembers hearing someone speaking rewind and replay the recording but with different words, or laughter, or someone else (a silly cartoon character?) speaking the words. At the end, stop the recording and delete it.*

TOOL: Smell Changer

Note

(Olfactory) Smell memories. Smell memories are very powerful although they may not be common except under certain circumstances (e.g. burning or decomposing bodies, gun smoke). . These intrusive olfactory memories seem to be particularly disturbing aspects of recall: they do not habituate easily, and are invariably accompanied by intense reliving of the emotions associated with the event at the time. Fortunately, it seems to be fairly rare that intense and distinct olfactory stimuli are encoded at the time of the traumatic event. The exceptions that stand out are when children have been close to rotting human corpses, or buried under dead bodies, or witnessed burning of people or bodies.

A simple and effective way of countering olfactory intrusions is to have to hand some pleasant aromatic herbs or oils. If possible, bring some to the group and hand them round so that children can take it in turns to smell them. Ask them to describe the smell as best they can. Ask them what it smells like, what it reminds them of, or what colour the smell seems to be.

Next, we're going to talk about smells that are connected with the traumatic event that might bother you. Sometimes, people feel like they smell all over again the smells that were there when they experienced the traumatic event. This is very frightening because it brings back all the scary feelings straight away.

If this happens to you, it's useful to have something else ready that you can smell to get rid of the bad smell.

> → ***Pass around*** *herbs/essential oils/different smelling items*
>
> → ***Ask children*** *to describe the smell; what it smells like, what it reminds them of, or what colour the smell seems to be, to choose a favourite and try to remember it.*

Even if you don't have something with you that you can smell, you can try to imagine the pleasant smell instead. Try to do this now - it's quite hard. Close your eyes. Try to imagine the smell of the oils we've just passed around. Remember what it was like, what it reminded you of, and what kind of colour it seemed to be. Can you do it? Good. Imagine you have the bottle by your side with the lid on.

Now, try to remember a really horrible smell - one of the most horrible things you've ever smelled. Imagine it as a small smoky cloud just in front of you. Really try to remember the smell. What colour is the smelly cloud? Have you got the smell? Good. Now watch it slowly shrinking in front of you. It's getting smaller and its colour is fading. Picture a balloon and watch as the smell gets sucked into the balloon as it gets smaller and smaller. Picture all of the smelly cloud inside the balloon. As it all disappears into the balloon, the smell fades away. Let go of the balloon and watch it float away taking the horrible smell with it. Now, to get rid of the nasty smell altogether. Imagine taking the lid off the bottle by your side. Bring back that nice smell with all its colours and reminders. Can you imagine the nice smell? Good.

Note

(Kinaesthetic) touch/movement memories. Re-experiencing sensations like falling, or of having been buried or smothered rarely occur on their own. They may improve when other sensory memories are targeted. They are best tackled through graded exposure in session 4 or with touch and massage (see parents' programme).

TOOL: Good Feeling Button

Note

There is another way that positive images can be used, called counter conditioning. Here the child is asked to recall different positive experiences from their life. It can be a good meal, a happy moment, a funny game, everything associated with pleasure and fun. Each time they are feeling the good, happy or pleasant feeling associated with this, they press one point on their body, i.e. the same point such as the top of their middle finger or their thumb. In this way this point will gradually be associated with the pleasant feeling. In a situation where they feel frightened or react to something that reminds them of the disaster, pressing this point will instantly trigger a good sensation that can counteract the unpleasant reaction (adapted from Paunovic, 2003).

Now here's another trick you could try. We've called it your Good Feeling Button. All you need to do is think of some nice times and feelings. It could be a good meal, a happy moment, a funny game, anything to do with pleasure and fun. Each time you get this or another good feeling just press on a special bit on your body, like your hand or the tip of your finger to help you remember it. You have to keep on doing it to make it happen automatically. Then you'll find that this special spot will become your own Good Feeling Button. So, whenever you feel frightened or are reminded of something unpleasant press on this spot to bring back the good feelings.

→***Consider*** *whether the children need a short break or stretch/move around/jump up and down before continuing*

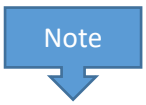
TOOL: Tapping Trick

Note

The dual attention tasks described next are derived from Eye Movement Desensitisation and Reprocessing (EMDR) techniques (Shapiro, 1995). The focus is again on intrusive traumatic imagery, but the methods used are quite different from the sorts of visualisation techniques practised above. Original individual treatment protocols for the EMDR dual attention tasks involved having children deliberately recall their traumatic image with their

eyes open while simultaneously tracking with their eyes the side to side rhythmical movement of the therapist's hand. However, research suggests that a number of alternatives to eye movements may be used with children as a dual attention task; and we have found that when working with groups, alternate tapping of knees works best. The technique is simple to teach, and may produce rapid spontaneous change. The basic technique involves each child tapping his or her own knee at a rhythm set by the group leader, while he or she holds the traumatic image in mind. Nothing else is required of the child, apart from to notice any changes to the image that occur spontaneously. Initially, three sets of taps for about 30 seconds each are carried out. A fourth set is then carried out while the group is told to let the picture fade. A final and fifth set is then carried out while the children are told to imagine a pleasant scene instead. The group is then asked for feedback; and the whole procedure may be practised twice more.

OK, let's make sure we are all sitting comfortably in our circle. Well done! You've all worked really hard and you've tried out some ways of changing picture memories and sound memories in your head to show them you're the boss. Now to finish we're going to show you a different sort of trick. This time we don't want you to change anything. We just want you to see what happens to your memories when you are doing some special tapping that we'll teach you. Just watch and see if anything happens. It may. It may not. Let's see. For this one you're going to need three things: your intrusive image/picture, a nice picture – you can use your Safe Place or choose another one - and last of all some special tapping.



The tapping pace should be brisk and regular, but not too fast because the whole group will have to follow your pace. Initially, to set the pace, count silently in your head, "one and two and three and four... etc"; each number about every second and each half beat on the half second. Tap roughly every half-second to set a brisk and steady pace that the children can all follow together. The speed of tapping is not crucial: more important is that a regular pace is set and that all children can follow it, so adjust the tapping rate as you

wish. It may be best to start slowly and speed up a little once all the children have got the rhythm.

OK let's sit in a circle on the floor or chairs. First, just watch me as I tap my knees to a rhythm.

Now you do it. Copy me as I tap my knees.

> →***Tap*** *knees to a regular beat using alternate hands…about 2 per second…to help set the pace count silently in your head "one and two and three and …."*

Now you do it. Copy me as I tap my knees.

Note

The second group leader might want to move around the group helping any child who has difficulty maintaining an alternating rhythm. Keep tapping long enough for everyone to join in. You may want to stop and start again to practise tapping without any image work first. Complete another half minute set of taps, and as before finish with the instruction to blank it out; and ask in a non-directive way what changes were seen.

You've really got it.
Well done, you can stop now. Let's try one more time just to make sure

> →***Tap*** *your knees in time with the children for 30 seconds.*

That's very good. Now stop.

Note

Moving on to deal with the unwanted image. This technique works through spontaneous changes occurring to the image during the tapping procedure. At the end of each set of taps give enough time to rest without the image before restarting. At the end of each set, ask what happened/did they notice anything in a non-specific way. Children may report a wide variety of spontaneous changes (to appearance, content, accompanying

feelings about the image) or nothing. Reinforce these changes in a non-specific way, but do not ask for details. Say something like "That's good. Just stay with those changes and see what happens next "

Now, I want you to bring up an image you want to take away or remove which upsets you. It doesn't have to be the worst, just one you don't really like.
Now keep your eyes open. Bring up the picture so that you can see it. Have you got it? Okay, now with your picture in mind and your eyes open, start tapping.
TAP

→***Tap*** *your knees in time with the children for 30 seconds. While they are tapping encourage them by saying something like ….*

….tap ….and just watch the picture in your mind ….and see what happens.

STOP. Blank out the picture. Good. OK, did anyone see anything happen to their picture?

→***Respond in a*** *non-specific manner... "OK", "Uh huh", "fine" etc.*

→***Don't ask*** *for detail*

Good. Some of you got changes, some didn't. That's normal. Just watch the picture in your mind as you are tapping and see if anything happens. OK? Let's try again. Bring up the picture you had when you stopped and TAP

→***Tap*** *for 30 seconds and encourage the children with occasional comments "Good", "Just watch what happens" etc.*

Now stop. Blank out the picture. Okay. What did you get?
Now stay with those changes. Watch the picture in your mind and TAP

→***Tap*** *for 30 seconds continue to encourage*

Now stop. Blank out the picture. Good.

We'll do the same thing once more. But this time, try to make the image fade away. Imagine you're turning the brightness down and watch as the image gets fainter and fainter as you carry on tapping. OK? Ready? TAP

→***Tap*** *for 30 seconds encourage the children to fade the image*

Stop. Blank out the picture. Good.
Finally, I want you to imagine a nice scene as you're tapping – maybe your Safe Place maybe something else. This scene will take the place of the bad picture. Bring up your nice picture, still with your eyes open. OK? Now, we'll start tapping once more. TAP

→***Tap*** *for 30 seconds encouraging the children with their nice image*

Now stop. Relax and well DONE.

→***Don't ask*** *about changes*

So now you've got a Tapping Tool and it will be really good to practice on your own. Maybe when you need to. But maybe just when you can. Just to remind you the steps are

1 Bring up your unwanted picture and tap.
  Just watch what happens.
  Stop and blank it out
2 Bring up the picture where you left off and tap.
  Notice what happens.
  Stop and blank it out
3 Carry on a few more times or until your picture stops changing
4 Finish by bringing up your nice image and tap.

Note

Just like intrusive recall while awake, bad dreams or nightmares are distressing and disruptive for children. When nightmares recur, children can become afraid to go to sleep at bedtime, or may not want to go back to sleep if they awake during a bad dream in the middle if the night. Lack of sleep in children can compound their irritability and poor concentration during the day, and schoolwork and peer relationships may suffer. Older children

may think it immature to have nightmares, and so avoid telling anyone: this avoidance is likely to maintain the nightmares. A number of techniques are available for tackling nightmares. Some of them are covered fully in later sessions with children or in the parallel session for parents, or have been covered in the sections above.

1. *Parents set up a regular evening bedtime routine (see session for parents)*

2. *Children practice relaxation techniques before going to sleep (see arousal session for children)*

3. *Children practice safe place techniques combined with relaxation before going to sleep (see section above)*

4. *During the day (not at bedtime) children recall their dream and practice changing the dream images or practice dual attention tasks while recalling the dream (see sections above)*

5. *Rehearsal relief: children draw, write down, or talk about their new and "positive" version of the dream in detail with a trusted adult (see below). They can change the bad dream in any way they like (see Krakow & Zadra, 2006)*

6. *Dream restructuring: children rehearse their new dream, and incorporate and practice a different positive ending, possibly with the addition of imaginary helpers or superheroes (see below). They can write the changed dream down. Before bedtime they say to themselves that this version of the dream must be chosen if they are to dream during the night. In addition they are to say to themselves upon going to sleep: "If I have a bad dream I will be able to hear myself say that this is only a bad dream" (See Spoormaker, van den Bout & Meijer, 2003).*

Lots of children have bad dreams or nightmares after they've seen something horrible. So, the next thing we're going to do is to show you some ways that you can take control of your nightmares.

Nightmares are very scary because they seem real when you're asleep. But they're not real. They're just dreams. They're just left-over stuff and worries. But they can make your heart go really fast, and make you feel really scared and horrible. The best way to fight back against them is to get them out into the open during the day when you're awake, when you know they can't hurt you. We know from talking with other children that if you get your dream out into the open in the day, then it will come back less and less at night- time. One of the best ways to get it out into the open is to draw it, and that is what we'll do now.

Note *Getting dreams out in the open.*

Now, let's spread out so that everyone can work on their own. I want you to draw your horrible dreams with all details. It doesn't have to be a good drawing – it's the dream that matters not what your drawing looks like.

→***Hand out*** *drawing paper and crayons. Allow time to complete the task*

Stop now. Well done. Now each of you show your picture. Let's go round the circle so each of you can tell us something about your dream.

→***Encourage*** *children to give more detail about their dream or drawing*

Note *Dream Rehearsal: Telling dreams over and over and setting up real life practice for between sessions.*

You've all done really well telling each other about your scariest dreams. One of the things to do between now and next time we all meet is for you to tell [mother or father or appropriate adult] about the dream. Tell them all about it, as much as you can. Remember, the more you talk about it during the day, the less it can come back at night. When you've told them about it, if you still feel scared or upset, practise going to your Safe Place to relax.

Note

Dream Restructuring: learning how to alter the content or ending of dreams. This section also draws on previously practiced skills with imagery and reinforces a sense of mastery.

Did you know that you can change your dreams if you want to? One way is to use some of the tricks we learned earlier, so that you could imagine putting your dream onto a television screen and changing it; or you could do the tapping exercise while thinking about your dream to see what happens to it. You could try this at home.

TOOL: Dream Planner

Another way to change your dream is to think of how you want the dream to turn out. Think about your dream now, and try to think of a different ending.

> →***Suggest*** *some examples, maybe using some children's dreams*

Think about what you would do in the dream if you were really powerful and could do anything. You could give yourself some magic powers. You could have your Imaginary Helper come and help you in the dream. Now I want you to think about a good story for the dream. A good ending - where you're in charge. And draw it. Right before you go to sleep tell your brain that if you are going to dream about this, this is the version to choose.

> →***Make sure*** *each child has paper and give time to draw and rehearse positive endings*

Stop. Well done. Now, who wants to show us their picture and tell us how they've changed their dream?

> → ***Allow*** *a few of the children to report back and* ***encourage*** *the other children to help each other with suggestions.*

Well done everyone. You've made some good new dreams. Maybe when you get back you can tell your (grown up) about your old scary dream and the new one you've planned for yourself. You can keep the picture by you when you go to sleep so you can use it if you need to, to remind yourself how your dream's going to change.

TOOL: Worry Time

Note

By learning to postpone worry children gain mastery over their intrusive, troublesome thoughts. We also recommend that a specific time period, e.g. 10 minutes a day, is set aside for these thoughts or worries. If the thoughts appear they just say to themselves that I am not going to worry or think about this now but later in the time set aside for this. They are instructed to acknowledge the thought and make a mental note of its content and let the thought fade at its own time. When the thoughts and worries appear, they can just notice them like clouds that pass over the sky without engaging in them. In the time they set aside to address their worries or thoughts they can write their thoughts down in detail and ask themselves questions about the reality of their worries (adapted from Wells & Sembi, 2004).

Sometimes it's not pictures or sounds that pop into people's minds when they don't want them to, but words and worries.

And worries are really only a thought or problem that bothers you which you keep trying to sort out. Some worries you can sort out on your own – like if you've fallen out with someone you can go and say "sorry". Some worries need someone else you know to do something – like finding a place to live. And there are some worries that you can't sort out. Sometimes it's possible to do something to take your mind off what's bothering you. That's useful because it helps you feel better. But it doesn't always help in the long-term. In fact, trying to ignore a worry can sometimes make it get bigger and bigger. And worrying takes up time and energy that we can use for other things.

So today we're going to give you a Tool to help you deal with worries – it's called Worry Time. And just like children have time set aside each day to go to school or to eat we want you to set aside 15 minutes a day to worry. You can do this until you have got control of the thought that's bothering you. During your Worry Time, you can think about what's worrying you. You can worry as much as you want. BUT you don't have to worry if you don't want to.

Here's what you do. If a bothering thought enters your head outside of your "Worry Time", just notice what it's about. Say to yourself; "I'm not going to think about that now. I'll think about that in my Worry Time". Let the thought

fade away as you start doing something else or start thinking about something else. Just imagine those bothersome thoughts are clouds floating high above you Just let them float about up there. They're not doing anything.
In your Worry Time write down the thoughts that came to you. Sort them into different groups - the ones you can deal with on your own; the ones other people need to deal with; and the ones that are left over. Then think what you can do about each bit. You might be able to deal with it on your own. You might need to ask for help to deal with it. Or you might have to let someone know there's a problem they should deal with.

So, don't forget when you find a worry pops into your head just say "I'm not going to deal with you now. I'll deal with you in Worry Time" and watch it fade as you do something different. If you do this every time the worry enters your thoughts, it will become automatic and the worries are stopped before you think about them.

And our next tool is called Doing Something Different.

TOOL: Doing Something Different

Note

Distraction: in the short term, this can be very useful but be careful not to encourage the children to deny or avoid dealing with what has happened. Giving the children permission to switch off from their traumatic memories is a helpful way to finish this session. It is also a way of bringing the group back together in what can be a fairly lively group discussion where they can show off their own coping strategies, and help each other.

We've talked a lot about the bad memories and worries you've all had because of the war. Some boys and girls say that some of the time all they want to do is just forget about what has happened – and that's OK. Tell me about the sort of things you do when you want to forget about the war or bad events. Maybe we can swap some ideas. Who wants to start? And let's make a list on the flipchart/board

→***Help*** *children have fun and think about activities they like doing which will distract them. You can add your own suggestions as well e.g. thinking about something in detail from beginning to end; Listening to or playing*

music, playing games, doing sport, reading a book, watching television etc.

→ ***List*** *on flipchart/board*

Well done. You've come up with some great ways of taking your mind off what has happened and what worries you. Even if it is only for a little bit. And Doing Something Different can be another good tool for your Toolbox.

Ending the Session and Homework

Note

The group is ended with the routine that was started at the end of the first group. Things to start with this time are

- *The favourite or most helpful part of the session*
- *The names of the tools learned so far*
- *The favourite tool learned today*

Homework to be done for the next session is to practise one or two of the tools learned about in this session – as well as continuing with Safe Place from the first session. The session is then ended with a pledge, song, fist bump etc.

It's the end of our second session now. Thank you for coming–it was lovely to see you all again. Once more you've all worked incredibly hard and talked about some really difficult stuff. So, very well done. And you've also got loads more tools in your Toolbox now, haven't you? Does anyone remember them all/some/one of them? Good.

→ ***Elicit*** *the names of the tools learned and, if it is helpful, list on the flipchart. Tools introduced in current session were Picture/Sound/Smell Changer, Good Feeling Button, Tapping Trick, Dream Planner, Worry Time and Doing Something Different. Safe Place was introduced in the previous session.*

Does anyone have a favourite? Or something else they found helpful from today?

→ ***Allow*** *anyone who wants to name a favourite tool or helpful part of the session to do so.*

Now for next time we would like you to have a go at practising one or two of the tools you tried out today. The more you practice these tools the better you will get at using them so it will be easy for you to use when you need to. And do keep on practising your Safe Place - and it is always there if you need it.

Now does anyone have an idea of a tool they would like to practise for next time? Good.

→ ***Allow*** *anyone who knows what they want to practise to say so.*

And before we all go, let's do one more thing.

→ ***Choose*** *from the list above: a pledge, sing a song, pass a fist bump etc.*

Session 3: AROUSAL, FEELINGS, RELAXATION AND COPING

As we saw in the Introduction, children, like adults, may show increased physiological arousal following exposure to traumatic events. So, children may be more nervous, jumpy, and anxious, and startle very easily. They may become more irritable and have difficulties concentrating. They may have difficulties falling or staying asleep. This can have an adverse effect on the child's functioning, in terms of his or her family and peer relations, performance at school, and sleep. There is also increasing evidence that this sort of increased physiological arousal is strongly linked to experiencing the sorts of intrusive recollections that were dealt with in the previous session. The direct negative impact of hyper arousal on the child's life, and its likely role in maintaining other symptoms of posttraumatic stress mean that it is an important area for intervention and prevention. The relaxation skills learned in this session are also important in preparing the group for the graded exposure work that will be carried out in the next section.



Feeling Thermometer
Relaxed Breathing
Short Relaxation
Mind Clean
Let it Be
Strong Thoughts
Healthy Sleep
Activity Diary

Materials needed:

- ✓ Soft ball (optional)
- ✓ Large sheet of paper or
- ✓ Body Outline (Appendix I)
- ✓ Pencil and coloured crayons
- ✓ Prepared post-it's or stickers for body feelings (optional)

Hello everybody. Welcome back.

Let's start by saying one nice thing you've done - or that's happened to you - since we last met. Well done.

Now, how did you all get on with your practising from last time? Did anyone talk to someone about their dreams? Good. Did anyone manage to plan a nice ending or change their dream before they went to sleep? How did that go?

And what about dealing with worries? How was that?

And what about the other tools – did anyone try the Good Feeling Button? The Tapping Trick? The Picture, Sound or Smell Changer? Doing Something Different? Good.

> → ***Encourage*** *the children to comment and* ***Name*** *each technique used as they are mentioned e.g. "Oh that's great, you tried the Good Feeling Button, Picture Changer, Safe Place etc..*

Note

Learning the connection between feeling scared and bodily sensations; helping youngsters describe their bodily sensations.

So, last week we talked about bad memories and things you can do to get more control over them, and you've all done really well in practising them at home.

This time, we're going to concentrate on what happens to our bodies when we're in scary situations, or when those bad memories come back. Remember that last time, we talked about how we can get jumpy and nervous when we hear loud noises, for example? Does anyone have that happen to them?

> → ***Elicit*** *examples e.g. I almost jumped out of my seat at school the other day when the door slammed! Use these to start brainstorming symptoms of increased arousal.*

Thinking about your body, what happens when you get really scared like this? What things do you notice happening to your body?

→ ***Write*** *reactions on a flip chart. Not everyone will experience the same so use this discussion to point out that they are reacting similarly and normally. Guide their attention if needed by saying things like …..*

So, what happens to your heartbeat when you're scared? What happens to your breathing?

Who said/did anyone say....?

→ ***Elicit*** *and provide most of the list below*

Heart – (racing/pumping/beating fast)

Chest – (pain/feels like it's going to burst/tight/out of breath)

Breathing – (shallow/faster)

Head – (spinning/racing/confused/dizzy)

Thoughts – (Scary thoughts/scared going to pass out or die)

Tummy – (tense/butterflies/feel sick)

Hands & fingers – (sweaty/tense/shaky/tingling)

Legs and feet – (tense/stiff; wobbly/tingling)

Body feels out of control, haywire, sweaty

Well done. Not everyone will have the same reactions. Sometimes people think there's something wrong with them or they're ill, like having a heart attack. Sometimes people can feel so bad they worry they might die. Has anyone here worried about what was happening when they felt these things happening in their body?

→ ***Discuss and Reassure*** *they won't die.*

Note

Learning the connection between what they feel in their body and feeling scared. Learning how thoughts and memories cause distress.

These are all horrible feelings, but they are what usually happen when we are scared or stressed. Our bodies are really clever and are built to get us ready for

action if there's a danger. It's the same for animals. They'll run away, fight or just freeze… to keep themselves safe. It's called the Fight, Flight or Freeze mechanism. And we feel Fear at the same time. These are very old survival instincts. The trouble is that the Fear reaction can be triggered by other things that remind us of danger. Thoughts and memories can do this. When one of these memories triggers our fear reaction, we can feel very uncomfortable wanting to Flee or Fight without always understanding where the feeling came from

The more scared we are, the more our bodies react; and the more our bodies react, the more scared we feel. So, a way to make ourselves feel less scared is to learn ways to make our bodies relax. IT'S HARD TO FEEL SCARED WHEN OUR BODIES ARE RELAXED. The more our bodies are relaxed, the more we can have good feelings and thoughts. So later today, we are going to learn ways to make our bodies relaxed.

Note

Grading feelings. Children need to be able to express how strongly they feel about something in order to monitor change and later on to create a way of tackling their fears.

But when you're learning to be relaxed it will be important for you to know how scared or tense or stressed you feel. Let's draw something up on our flipchart/board that can be used to measure things.

> → ***Elicit*** *suggestions e.g. jug, thermometer, ruler etc. Use the thermometer as below or choose one suggested by the group*

TOOL: Feeling Thermometer

I'll use a thermometer. At the bottom, number 1 means you're not feeling scared or stressed at all. Can you think of any other words you could use to show that you're not at all scared?

> → ***Draw*** *a thermometer on the flipchart/board*
>
> → ***Elicit*** *descriptions suggestions e.g. relaxed, calm and write some at the bottom of the scale with a 1*

→ ***Fill in*** *a scale from top to bottom with the agreed numbers and verbal descriptions*

At the top is the most scared or tense you've ever been. What words can you use to describe the top of the scale (e.g. terrified, too scared to move)?

→ ***Elicit*** *descriptions to anchor the top of the scale and write this at the top with an agreed numerical rating*

→ ***Repeat*** *this for different points on the scale so that children get used to rating their feelings*

Each of can make your own Feeling Thermometer to keep.
When you practise relaxing your body, you can use your Feeling Thermometer and notice how the rating goes down and you feel calmer and more relaxed.

And remember, if you feel so tense or frightened that your feelings are at the very top of the scale, you can always go to your Safe place to help you feel better. Your Safe place is yours and it is always there for you.

Note *Valuing the children's own strategies for reducing tension.*

What do any of you do when you feel really anxious, or uptight or tense [or however you describe it these days]? {Elicit a number of methods and reinforce those that are useful.}

→ ***Elicit*** *their own coping strategies. Some may be part of their cultural practices and not recognised for what they are e.g. meditation, yoga etc.*

Some of you already have good ways of getting relaxed quite quickly. We are going to show you some other ways of relaxing when you need to. But remember relaxation is like learning any skill - you need to practise to get better. The more you practice it, the easier it will come.

Breathing is important during relaxation. Sometimes, when we're scared, we take quick shallow breaths, and begin to feel out of breath. Now we're going to

learn a way to breathe which stops us taking short little quick breaths, and helps us to relax.

TOOL: Relaxed Breathing

Note

Reducing arousal by teaching diaphragmatic breathing. This helps reduce over-breathing and its consequences e.g. feeling faint etc.

Everybody place your hands on your stomach, just below your ribs. Breathe in slowly through your nose all the way down to your stomach. Feel your stomach push out. And now blow out slowly through your mouth. Let all the air come out and feel your stomach go down. Now, I'll count so we get a rhythm going.

> → ***Set a pace*** *by counting out loud to 4, one beat per second. In the following exercise, the instructions to Breathe "in" and "out" are substituted for the number 1 but keep the rhythm going. In the second part of the exercise say "Relax" instead of "out", again keeping the rhythm going.*

Breathe **in**…2..3…4 stomach up ..and **out**…2…3…4 stomach down…breathe in in…2…3…4…and out...2…3…4…breathe in…2…3…4.stomach up…and out…2…3…4 stomach down. Well done.

Now we'll do that again but this time, as you breathe out say quietly to yourself "relax". I'll start you off by saying it then you can carry on without my help. Hands on tummies. Breathe in…2…3 …4and out…2...3…4Breathe in…2…3…4 Relax…2...3…4 and in...2...3 ...4… Relax…2…3….4…Breathe in…2…3…4..Carry on …2…3 …4…

> → ***Wait quietly*** *for a few seconds whilst the group continue on their own*

Well done. How was that? Did anyone find it difficult or have things pop into their head? You can use this special breathing before you go to sleep or with other things like your Safe place. We'll use it again in our next technique.

TOOL: Short Relaxation

Note

It is useful for children to have at hand a short form of relaxation that they can use quickly in an anxiety-provoking situation.

First breathe like you just have learned. Put your hands on your stomach. Breath in slowly through your nose counting slowly to three, then blow out slowly through your mouth (counting slowly to three), saying to yourself, "relax". Do this four times. Then first tense and then relax one fist and then the other while you count to five while tensing and then again while relaxing. Then do the same with your legs, first tense them while you count to five and then relax while counting to five. Then take four more breaths in through your nose and out through your mouth with your hands on your stomach. Say "relax" when you breathe out. Now picture yourself holding all tension or anxiety in your fist. Let go of the tension as your fists are relaxed. When you've finished relaxing your fist shake it to get rid of those last bits of tension or anxiety.

Note

The following techniques are optional for leaders to teach and children to try out. The message is that most kids find some of these techniques helpful, but not all techniques will work for everyone. The use of imagery is very powerful. By thinking in particular ways, it is possible to influence our body. Here the children learn to reduce stress using imagination. This also reinforces their sense of control over intrusive images.

Over the last few sessions we've seen how our thoughts can affect our feelings. For example, in Michael's story, he did not like to think about what he saw because it made him feel afraid. We've talked about how thoughts can affect our behavior. When Michael thought about the killing, he did not want to go outside. Next, we are going to concentrate on how we can change our thoughts so that our feelings and behaviour change too.

TOOL: Mind Clean

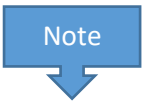
Note

If the example below is not culturally appropriate let the children imagine they are pouring water over their head and that the water washes away all unwanted thoughts, tension or anxiety.

Find a good place to sit in. Not too close together. Now the next Tool we've called Mind Clean because just like a house or room needs cleaning to get rid of

dust and left-over rubbish, our minds need cleaning of unwanted material and tension. Close your eyes. If you can't, find a spot on the floor in front of you to look at. Now, imagine that you've got a sponge in your hand. Is it wet or dry? If it is wet, twist it until it is as dry as possible. Now lift it up and hold it to your forehead. Now, imagine that all the tension, worry and anxiety are pouring out of your mind and into your sponge. Let it pour out for a minute or so. Then lower the sponge and twist it until all unwanted tension and anxiety is gone and then lift it up to your brow again. Imagine that any tension/worry/anxiety that is left in your mind is poured into the sponge. If you want to you can imagine a strong pump sucking whatever tension is left into the sponge. Once more twist the sponge until it is completely empty. Feel how good it is to have gotten rid of all that unwanted stress and tension. You can use this mind-cleaning as often as you need.

TOOL: Let it Be



The following technique uses a focus of attention and a particular, non-judgmental attitude. This technique develops the state of mind the children were asked to adopt in previous exercises - Tapping Trick and Worry Time.

Now we'd like to add another tool to your toolbox that is easy to learn and that you can do anytime throughout a day. We've called it "Let it Be". It is based on you just noticing what is happening inside or around you. It is important that you accept what happens and do not get angry at yourself if your mind starts to wander. It will and you can just notice it. That is what this is about, just noticing things. For the next minute I want you to focus all your attention on your breathing. Just notice what happens when you are breathing. Breathe as you normally do, but just focus all your attention on your body. If your mind wanders off, just notice it and return your attention to your breathing. Just experience what happens when you turn all your attention to your breathing. Do not judge what happens, just notice it. Practice this focusing of attention often and learn to be in the present moment. You may do the same when you hear a piece of music or turn your attention to the sounds and sights around you. Focus your attention on one thing at a time and just notice everything about that thing, be that a special sound, a beautiful sight, or a good sensation in your body. This focusing helps your body relax and you can then focus on this good feeling in the same way. Just notice it.

TOOL: Strong Thoughts.

Note

Positive self-talk can be a useful relaxation technique for some children. They must first understand the connection between thoughts and feelings, then be able to identify anxiety-provoking thoughts, and last be able to substitute more adaptive, positive, coping enhancing thoughts. This activity may best be suited for older groups. With younger children, it may be possible for them merely to generate a list of positive things that a helper - real or imaginary - might say to them if they were in the anxiety-provoking situation. Although conceptually hard at first for some children, this section can be made into a fairly brief and fun activity.

Something that can make us feel worse is the thoughts and things that we say to ourselves when we are scared. In fact, thoughts, feelings and behavior all affect each other. For example, just say you were in bed at night and you hear a noise in the house.

If you think the noise is because someone has climbed in through a window how would you feel?

→ ***Elicit*** … scared, anxious etc

And how would your body feel? What might you notice in your body?

→ ***Elicit*** … heart racing, tense etc

And what might you do?

→ ***Elicit*** *… hide under the bed, call the police etc*

Another night, you might hear the same noise. This time, you think it's just the wind or a cat that you like climbing in. How would you feel if you thought that?

→ ***Elicit*** *… emotions other than anxiety and fear*

And what would you do?

→ ***Elicit*** *… go back to sleep*

So, you see thoughts we have and the things that we say to ourselves can make us feel worse or make us feel better. And they can also affect what we do.

What sorts of things go through your mind when you get scared or are worried about doing something –like if everybody's depending on you to score the winning goal (or some other appropriate example)?

→ ***List*** *… on flipchart anxiety provoking thoughts such as*

- *I can't do it*
- *I'll lose control*
- *I hate this*
- *I'm no good*
- *This is worse than it's ever been*
- *I'll panic*
- *It's all coming back on me again*
- *I can't cope*
- *My fear will never go away*

What POSITIVE things could say to yourself instead? Or what might your [parent/elder sibling/teacher/football hero] say to themselves if they were in your situation?

→ ***List***… *on the same flipchart positive self-statements that they can say to themselves instead*

- *I can do it*
- *I know I can cope*
- *This can't really hurt me*
- *I know I'm brave*
- *I won't let it beat me*
- *Memories can't hurt me*
- *It'll be over soon*
- *Just stick it out*

So you see, the sorts of things you say to yourself can make you feel better or make you feel worse. They can help you or undermine you.

Now each of you write down the 3 or 4 you like the best on a piece of paper. You can keep this with you and read them or just remind yourself they are there when you need to. We'll call these your STRONG THOUGHTS because they'll help you to feel better and help you cope. Try to use them before the next time we meet and we'll see how you get on.

Note

Optional exercise. Learning to swap thoughts which make you anxious for more adaptive, coping ones. Through this exercise the children are shown, in a fun way, how what they say to themselves can help them do things they find difficult.

Now let's practise with another tool for your Toolbox. This time we're going to practise swapping unhelpful, scary thoughts for helpful, STRONG THOUGHTS. First of all, we need something to be scared about. Let's choose one of the reminders from the list that we made in our first session.

→ ***Elicit*** *and elaborate a situation with the group e.g. going past a building that was bombed*

Now who wants to volunteer to pretend to be scared? You're going to be telling us all the scary, unhelpful things that are going through your head as you face the reminder. And the rest of us are going to help you by shouting out some helpful strong thoughts to say to yourself instead.

→ ***Encourage*** *the children to join in using statements like the ones on the flipchart. This should be a fun session.*

Great.

TOOL: Healthy Sleep.

Note

Sleep difficulties due to over arousal are common in children who live through war. The main strategies for tackling these are: establishing a regular night-time routine, dream restructuring and relaxation before bed. Children may also have their own strategies.

Lots of children who are stressed have trouble sleeping. But getting a good night's sleep is important for all sorts of reasons. Does anyone here have, or has anyone here had problems in the past with sleeping?

→ ***Encourage*** *a discussion.*

→ ***Identify*** *the type of problem – getting off to sleep (worry/tension); waking in the night (bad dreams) or waking very early and not getting back to sleep (not common). Severe disruption of sleep may indicate depression*

What sorts of things did you do to beat your sleep problems?

What else can you do to get a good night's sleep? Let's write your ideas up on the flipchart.

→ ***Encourage*** *a discussion.*

→ ***List their suggestions*** *on flip chart to include the items listed below.*

Note

Ask what the children did about their sleep problems if they managed to beat them. Ask the rest of the group to make suggestions to help them. Children may come up with some of the following themselves. If not, bring them in to the discussion and advise children of the following.

1. ***Develop a regular bedtime routine****:*
   This will depend on local practices. Try to have a wind down period or quiet time in the lead up to bed. Parents group covers this and will help them. Check on sleeping arrangements. Younger children may prefer to sleep in the same room as an elder sibling if they tend to get anxious in the night. Be aware that situation may have many sleep in same room (shelters, tents).

2. ***Reduce things which are likely to keep them alert before bedtime:***
   Older children should stay away from caffeine and nicotine in general, but before bed in particular. For younger children, coca/Pepsi cola can have same effect. Try to avoid internet and mobile phone use.

3. ***Prayers*** *before bed are very helpful for many children, and can help them to feel safer.*

4. ***Encourage relaxation practices*** *(muscle relaxation) before bed, every day.*

5. ***Do things to help them fall off to sleep****: once in bed, children may like to hear stories read to them, or like to read to themselves. Reading or playing games on computer devices are best avoided. Other children*

may like to listen to music to help them drop off to sleep. Sometimes, having the volume very low on the radio means that children need to concentrate hard to listen, and thereby are not distracted by intrusive memories or anxious thoughts.

6. ***Dream work**: encourage children to tackle dreams in this way. Some children with a history of nightmares may fear dropping off to sleep, but this should diminish once they start to work on the nightmares.*

TOOL: Activity Diary.

Note

Children's often stop doing activities after traumatic events. It is often important to re-establish them in a structured way, through planning and record keeping. It is very important that children are given permission and encouraged to do things they enjoy. There are many benefits to re-establishing activities including showing children that aspects of normal life are possible even in war; it encourages hope for the future, good self-esteem and sense of achievement. Playing with peers and sports activities, promote general development and children are less likely to worry when they are involved in something. Physical activity is known to reduce tension and help to lift depressive feelings and also children who are active during the day will sleep better at night.

Help the children make a list of as many activities as possible, not discounting any suggestions. Where appropriate, make reference to school-based activities or groups, all youth clubs, or groups. If NGOs or other groups are working in your area, make sure you know what they are doing beforehand, and refer to any activities that they are putting on for children, such as music or sports groups. Very ordinary activities should be encouraged, too, such as "playing with my sister", or "going out with my friends", but encourage children to be as specific as possible. This session should be fun and uplifting.

Doing the things you like doing during the day can help with sleep. But sometimes, when we feel anxious, afraid, or sad, it's hard to get back into the things we enjoy, but it's good for us to do as many of them as we can. One way of winning back our favourite things is to plan what we're going to do, and then to stick to the plan. We're going to keep a diary of all the things that we've done, so that we can look back on it and see how well we're doing in beating our Post Traumatic Stress.

Now, I want to write on the flipchart/board all the things you do – or used to do - for fun in your spare time. Who wants to start?

> →***Elicit*** *activities children enjoy. Include obvious ones such as football but don't forget simpler ones like playing with my sister etc.*
>
> → ***Hand out/help children prepare*** *a simple diary or sheet of paper divided into days of the week.*

Well done. Now you each have your own diary sheet. Everybody think of one nice thing you want to do each day from the list, and write it down or draw it on your sheet. There may be things you used to do but can't now because of the war – it mightn't be safe to do so or maybe the equipment has been destroyed. And don't forget you can add anything that's going on in school or that's been organised for you in the area. Do be as specific as you can – don't just write "do sport" say what and where. And choose something that's possible to do.

Add in your diary when you've done things, so we can look back on it when we meet next time.

Ending the Session and Homework

Note *The group is ended in the same way as previously.*

It's the end of our third session now. Thank you again for being here. Once more you've all worked incredibly hard and talked about some really difficult things. Well done everyone. And you've also got even more tools for your Toolbox now.

We can just go through them again to help you remember so you can choose what to practise. Today the tools were…? Who would like to start? And the other tools were…?

> → ***Elicit*** *the names of the tools learned and list on the flipchart.*
>
> *Tools introduced in the current session were Feeling Thermometer, Relaxed Breathing, Relaxation, Mind Clean, Let it Be, Strong Thoughts, Healthy Sleep and Activity Diary.*
>
> *Tools introduced in previous sessions were Picture/Sound and Smell Changer, Good Feeling Button, Tapping Trick, Dream Planner, Worry Time, Doing Something Different- and Safe Place.*

Does anyone have a favourite? Or something they found helpful from today? For next time have a go at practising one or two of the tools you tried out today – and any of the others from the previous sessions. As we keep saying, the more you practise the tools the better you will get at using them and the easier it will be for you to use when you need to. Now does anyone have an idea of a tool they would like to practise for next time? Good. And before we all go, let's end as we usually do.

Session 4: AVOIDANCE AND TRIGGER ACTIVITIES

As we saw in the Introduction, and in the case example from Section 1, avoidance phenomena are cardinal symptoms of Post-Traumatic Stress. Avoidance may be cognitive (that is, attempting not to think about and remember the traumatic events); or it may be behavioural (that is, attempting to avoid reminders of the traumatic events, such as places and people; and avoiding talking about the event). Avoidance phenomena are legitimate targets for intervention for two main reasons. The first is that avoidance may entail restrictions on the child's functioning: it has a primary and direct impact on his or her life. The second is that although avoidance may provide some temporary short-term relief, in the long term it only serves to maintain the child's problems.

We saw in the Introduction how intrusion and avoidance symptoms can form a vicious cycle. The first session on intrusion aimed to break this cycle by teaching children how to control their intrusive memories. This section on avoidance aims to break the vicious cycle by giving children the skills to deal with avoidance



Fear Ladder

Materials needed:

- ✓ Worksheets/booklet
- ✓ Paper
- ✓ Crayons/pencils

Hello everybody. Welcome back. We all know each other now, so let's start by talking about how you all got on with your practising from last time?

→ ***Encourage*** *the children to comment and* ***Name*** *each technique used as they are mentioned e.g. "Oh that's nice, you tried Relaxation/Strong thoughts/Mind Cleaning/Safe Place".*

Did anyone else have a go at Short Relaxation/Strong Thoughts/Mind Cleaning etc.?

> → ***Ensure*** *you have covered each one. Feeling Thermometer, Relaxed Breathing, Relaxation, Mind Clean, Let it be, Strong Thoughts, Healthy Sleep and Activity Diary.*
>
> *Tools introduced in previous sessions were Picture/Sound and Smell Changer, Good Feeling Button, Tapping Trick, Dream Planner, Worry Time, Doing Something Different- and Safe Place.*

Well done.

Note

In some ways, this is the hardest of the three sections for children because it involves deliberate confrontation of traumatic memories. This is why the activities of today's group build on the control and relaxation techniques learned in the first two sessions. The aim is not to conduct an intense group exposure session, but to educate children and prepare them to carry out imaginal and in vivo exposure themselves afterwards with a trusted adult (see Parents' Group notes). Care should be taken not to reinforce partial exposure here. Remember that in general, children may not want to talk about what has happened because:

- *it is too painful*
- *they lack the words or concepts to express what they feel inside*
- *feelings and emotions are contradictory*
- *they feel guilt and confusion*
- *they are afraid of not being heard*
- *they are afraid of losing control*
- *they need time to build up trust*

- *they do not feel the need to talk, or feel that they have talked enough already*
- *they have others they talk to*

Initially, it is helpful to talk about reminders and behavioural avoidance since this is more concrete and easier for children to relate to. This is also part of building their motivation to confront and fight their fears. It is particularly important in situations of an ongoing disaster to have children distinguish between adaptive and maladaptive avoidance. Following teaching of skills related to behavioural avoidance and the introduction of the ideas of hierarchies and graded in-vivo exposure, the second of the sessions will be devoted to cognitive avoidance and the teaching of writing and drawing tasks to counter it.

Over the last three sessions, we've talked about bad memories and things you can do to control them. We've talked about how your body feels when you get scared, and things you can do to control your body feelings. You've all learned things to do, and you've got a Toolbox of tricks and special things to do to take back control of what you think and how you feel. Now you're really well prepared to beat your Post Traumatic Stress. Today, we're going to show you one of the best ways to beat it –and that's to face up to it.

Note

By talking about reminders, the children are starting to face up to the things they are avoiding. Remember that open discussion of reminders is a kind of exposure, and should be handled sensitively

The first step in facing up to it is to be aware of what brings back bad feelings for you.

→ ***Show*** *the list of people and situations from the first session.*

Remember in our first meeting that we made a list of reminders. Let's look at the list again on this chart. Reminders are things that can trigger upsetting memories. We tend to stay away from reminders so that we don't get upset or

frightened. Who wants to tell us about a reminder THAT THEY TRY TO STAY AWAY FROM? Who wants to add something else to this list?

Remember – it won't be the same for everyone.

> →***Elicit*** *as many as possible and get examples of the most problematic ones (e.g. the place where the event occurred). E.g. in war*
>
> ***places****: destroyed buildings, shrapnel or shell marks on buildings* ***and*** *streets; graveyards &, burial sites*
> ***people****: soldiers, police or militia; men in uniform or carrying weapons; injured survivors*
> ***situations****: separation (triggers fear of death); being alone; family gatherings, mealtimes; a time of day or day of the week; church or religious service;*
> ***sounds****: gunfire, mortar fire; shouting, screaming; sudden loud noises; sirens, air raid warnings; cars*
> ***smells****: burning; dead bodies*

And are there other things – like people or places or special times – that set you off, and THAT YOU WANT TO STAY AWAY FROM? Or maybe things you are frightened of doing because of the war?

Note

Some children might give examples of dangerous places that they avoid, like damaged buildings. Be sure to point out the differences between helpful avoidance of dangerous places (which keeps children safe) and unhelpful avoidance of places which are no longer dangerous (which does not make children any safer, but keeps their fear going).

There are many different fears a child can experience following a war. One is the fear of reminders that we have just described, but children may develop a fear that loved ones will die, especially if they suffered during the war. They can therefore develop separation anxiety and do not want to be away from loved ones, or may be preoccupied with their safety when at school. The

principles children learn in this session can be used to reduce the fear of reminders and to confront other fears, such as separation anxiety. As a group leader, you will motivate the child to work to reduce fears that in some ways limit their life. That may mean the fear of separation for one child, while it may be fear of a reminder that makes them stay away from play activities for another child.

Good Avoidance

Sometimes it IS a good idea to stay away from places – or things and people – because it's not safe. Maybe there are some safety programmes for you in school – like Mine awareness? When do you think it might be a GOOD idea to stay away from something?

→ ***Elicit*** *ideas…*

So, if you want to go somewhere or do something remember the safety programmes and think to yourself:

1. Is this safe?
2. Do adults come here?
3. Would an older brother or sister or friend come here?

If the answer to any one of these is NO – DON'T GO. So remember, NO- DON'T GO!

Note *Creating a personal list of reminders that the child avoids. These will be used later.*

Good. Now, I would like you each to make your own list, like we've just done on the flipchart, of things THAT YOU AVOID because they remind you and make you frightened. Choose things that are not really dangerous, but you stay away from them, or have stopped doing them because they trigger upsetting memories and make you frightened/set you off

→ ***Hand out*** *paper and pencils and walk around checking the children are putting safe examples down….*

Note

Helping children see that fears are common and that there are ways to get over them. Most children probably already know that one way of overcoming fear and avoidance is to confront the feared object or situation. Get them to say this early on.

That's great. Now you each have your own list of things that YOU stay away from because they trigger upsetting memories and make you frightened. We are going to show you some ways to fight your fear of these reminders.

But first let's talk about things that people are sometimes scared of, things that are NOT RELATED TO WAR. When we're afraid of something, we feel scared inside, and our bodies feel nervous and shaky, so we usually try to get away from the thing we're scared of. We call this avoidance. So, people who are scared of spiders, or dogs, or heights, for example, try to avoid them. Has anyone here had that happen to them, or know someone else [not in the group] who that has happened to?

→ ***Elicit*** *examples not specifically related to war.*

So, if you avoid things that make you frightened or set you off you feel better for the time being. But, what happens the next time you come across it? That's right. It sets you off again. So, although it stops you feeling bad in the short term. In the long term, it doesn't help.

And I want to tell you a story about a girl who was afraid of high places, any high places - even safe ones. Let's call her Susie.

Susie's Story

When she went somewhere high up, like a ladder, or onto a balcony or flat roof, or even along a path with a steep slope on one side she felt really scared. To stop herself feeling scared, she would avoid going up anywhere high. This

made Susie feel better, but it meant that she couldn't do some of the things she wanted to, like climbing or going up on a swing or even visiting her friend who lived in a block of flats. In the end, she decided to beat her fear of high places. How do you think she did it?

→***Elicit*** *a couple of ideas and try to get the group to come up with the idea of deliberately putting herself in the situation that frightened her. Whether or not they come up with confronting the situation a little at a time, continue to go through the steps in Susie's Story.*

Well done. Good thinking.

Note

Introducing the idea of Facing Your Fears in small steps. The first step should always be something that isn't too difficult. At each stage the child is instructed to remain in the situation until their anxiety reduces. It is therefore important not to make too great a leap from one step to the next. The following story includes each of these features.

Susie knew she had to get used to going higher and higher until she wasn't scared anymore. The important thing was that she did this bit by bit. She made herself a plan. To start with she made herself go up the first set of stairs in her house and stood there until she felt ok. That was easy because it wasn't much of a problem for her in the first place. Then she realised that there were other things that made it hard for her to go anywhere high like looking down to the ground when she was up there. So she added those to her plan too. She made herself look out of a window and even though she felt scared at first, she stayed there, and in the end her fear went away. She added things to her plan until she was able to go right to the top of her house and look out of the window without being scared any more. She was really pleased with herself for beating her fear and the next time her friend asked her round to play Susie could go and didn't miss the fun!

So, one trick to fight your fears, like Susie did, is to make a plan and build yourself a Fear Ladder.

→***Draw a*** *ladder on the flip chart so everyone can see. Using Susie's example below point out the positions on the ladder as they occur.*

Susie's goal was to be able to go up high places – like to the top of her house – so this would go at the top of her ladder. At the bottom of her list was going up the first set of stairs, as that wasn't much of a problem. She made a list of all the things she would have to be able to do to get to the top. Next, she put these in order from the easiest right up to her final goal and then she had her very own Fear Ladder with all the steps!

TOOL: Fear Ladder

Note *Here we are using an example taken from Michael's story to show how to build a Fear Ladder .*

Now let's do another example together from Michael's Story and this time we'll write the steps on the ladder.

Michael wouldn't go to school anymore because he was too upset to go to the place on the street where he had seen men being shot - even though he knew that all the soldiers had gone and the street was safe. So his goal is to be able to walk through the street he has been avoiding. At the TOP of his ladder we put – Being in the street where he saw the shooting. .

→ *Write on the ladder – using the children's suggestions – each step in the appropriate place.*

Note *Sometimes it can be too difficult to physically approach something. If this happens the task can be broken down into even smaller bits by first looking at the object until the bad feeling goes away. You can also use pictures of the object or even ask the child to picture the object in their mind. If the child is*

avoiding doing something imagining carrying out the action in mind is a good way of starting.

As in Susie's Story there may be more than one type of thing that triggers the child's fear reaction e.g. visual similarity (size, colour, uniform), auditory triggers (sirens, male voices, language). You will need to add these in a graded fashion to the fear hierarchy.

Now we have to break down what Michael has to do into small steps. What do you think would be on the bottom rung of the ladder? Something that's not too difficult. What about standing outside his front door thinking about going to school?

→ ***Elicit*** *other steps on the ladder putting them in order from easiest to most difficult. Use the strategies mentioned in the Note above if needs be. The steps might include things such as:*

- Being at the far bottom of his street, not looking towards the spot

- Looking towards the spot he's most frightened of

- Taking the first actual step towards the spot

- Being 50 metres away

- Being 10 metres away

-Being exactly where it happened

Good. Now you've got steps for Michael's ladder. Think about your Feeling Thermometer to check that the steps are in the right order and whether we need to add in some extra ones so that nothing is too hard for him.

Now you are all ready to make your own ladder to help you face one of your fears and a thing you are avoiding.

Choose one thing from the list you made earlier. It shouldn't be something that will be too difficult for you. And remember it should be something that's safe, but that you stay away from because it makes you feel frightened. You've stayed away from it so far, but now you feel ready to face up to this fear.

→ ***Hand out*** *paper and pencils so that the children can draw an individual ladder*

Draw your own Fear Ladder. Put your Goal at the top. Now think of all the steps you'll need to get there. Use your Fear Thermometer to help you to put them in the right order. You can move them around until you think you've got it about right. You can always change the order later if you find you need to or even add some extra steps. Any questions about how to do it?

→ ***Walk*** *around to help them. Make sure the steps are small.*

→***Allow*** *enough time (up to 10 minutes) and help finish some if needs be*

Note

Preparing to practise in real-life. The children need a set of simple instructions about how to do this. Children can be told that adults who have been to the parents' group know how to help. Children should be told that they can practise themselves if they feel able, or they may choose to have an adult help them start.

Now that you have your fear ladder, you're ready to face up to your fear. Remember that this will be a little bit scary at first, but you will take very small steps up the ladder so you won't get too frightened. It's entirely up to you how quickly you go up the ladder but don't forget to wait long enough at each step to let your fear go down. Only go up a step when you're ready. You have all the tools you need from previous sessions to help you cope, and your parents or another adult can be there at the beginning to help if you want. What things could you use from your toolbox to help you fight your fear?

→ ***Elicit*** *techniques from the Toolbox – relaxation and breathing tools, Safe place, Feelings Thermometer and Strong Thoughts*

That's right - you can use your relaxation or breathing tricks to get calm, your Feeling Thermometer to check that your fear is going down. It doesn't need to go down to the bottom, a 2 or 3 should be good enough. And Strong Thoughts to get you through it.

Let's just remind ourselves of the sorts of things you can say to yourself when you feel scared or worried about doing something?

→ ***Elicit*** *statements e.g.*

I can do it

It can't really hurt me

I know I can cope

I'm going to beat my fear

My fear will go away soon

You can always choose other ones to suit the situation.

Note

The children are now able to put together their own plan of action to tackle avoidance. By reviewing the stages you are giving the children a structure they can adapt to other situations.

Putting it all together

Now you're ready. Here's how to begin facing up to your fears.

→ ***Write/read out/elicit*** *the following instructions on flipchart.*

Or

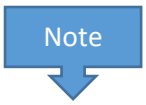
→ ***Hand out*** *prepared instructions or paper for children to copy down*

1. Choose a target
2. Make your Fear Ladder
3. Prepare yourself for the situation on the first step (tell yourself you'll do it even if it's scary).

4. Take the first step:
    - Calm yourself using Relaxation, Breathing, Strong Thoughts
    - Give a number to your fear on the fear thermometer
    - Wait and watch how your fear goes down by itself (if you get scared, remember your relaxation, practice your breathing and use Strong Thoughts).
    - Stay in the situation until your fear goes down to a 2 or 3
5. Be really pleased with yourself, and reward yourself
6. Move on to the next step

Well done! You're on the way to beating your Post Traumatic Stress and winning back the fun and normal things you want to do.

Ending the Session and Homework



The group is ended in the same way as previously.

It's the end of our fourth session now and the next one will be the last. Thank you again for being here. You have all done really well. Today you added one more tool for your Toolbox, setting up and using a Fear Ladder. For next time have a go at making and trying out your Fear Ladder – and do get someone to help you try it. Don't forget the other tools you know to help you if you find taking any step difficult – and do add an extra step if the jump to the next seems too difficult. Of course, you can practise any of the other tools as well.

***Choose** a pledge, sing a song, pass a fist bump etc. to end.*

Session 5: AVOIDANCE: MEMORIES

In the section above, we noted that in the short term behavioural avoidance is often helpful; but that in the longer term, deliberate and paced exposure to traumatic reminders and triggers is usually necessary. Similarly, in the first section, children were taught means of distracting themselves from their traumatic memories or anxious thoughts. This kind of distraction is a form of cognitive avoidance, and can be helpful in the short term as a means of regaining a sense of control. In the longer term, however, as with behavioural exposure work, it will usually be necessary for children to deliberately expose themselves to their traumatic memories. In effect, by talking and thinking about the disaster and children's memories of it in all sessions, a good deal of cognitive exposure has already been achieved.

The following section contains advice and activities for children to deliberately recall their traumatic experiences and, rather than changing or distracting themselves from them, to enable themselves to re-experience them in a structured and controlled way. These sorts of activities are powerful means of dealing with Post Traumatic Stress because they promote emotional processing and integration of traumatic memories

Depending on the children's age, level of maturity, and educational experience, writing is often a most effective means of processing memories. For older, as well as younger children, drawing is also very helpful. Where the technology is available, talking their experiences into an audio recorder and then playing it back later can be useful. Last, for children and adults alike, talking about the event with a trusted adult is helpful when done with a good listener. Choose one or more of the following activities as appropriate.



Drawing
Writing or Recording
Talking

Materials needed:

- ✓ Soft ball

- ✓ Worksheets/booklet
- ✓ Paper
- ✓ Crayons/pencils

Hello everybody. Welcome back. Today is our last group. You have done so much in the time we have been meeting. Let's start today's session by talking about how you all got on with your practising from last time? Did anyone have a go with a Fear Ladder? That's good/ ok you've not tried it yet – but you know what to do if you need it later. Did anyone try anything else?

> → ***Listen to and approve*** *homework efforts.*

The very last set of things that we're going to do involves deliberately thinking about your traumatic memories. The aim is the same: for you to beat your Post Traumatic Stress so that you can spend more time thinking about and remembering good things. Just like facing up to reminders and things around you, facing up to your memories can be scary, but you've all shown how brave you are already, and you all have a tool box of tricks and special things to do that will help you to be in control and beat it.

Note *Avoiding thinking about what has happened. Normalising and learning limitations of putting things out of your mind.*

When we talked about bad memories in the first group, we said that often we try to push them out of our mind because they are too upsetting to think about. This is a bit like avoiding places that remind us of the traumatic event because it's too upsetting to go there. Although pushing the memories away makes us feel better for a short while, in the end, the memories will probably just come back again later. Does anyone in the group have this experience?

> → ***Ask*** *for examples and encourage discussion using questions like those below.*
>
> → ***Allow*** *the children to avoid giving details of their memories. Avoidance is ok at this point. It will help them feel in control and enable them think.*

How successful are you at pushing the memories away?

Does it always work? What happens in the end?

Note

Reducing the power of memories. Recognising that pushing memories away doesn't usually work in the end and suggesting an alternative way of dealing with them. Getting them out in the open. Give children permission here to avoid thinking and remembering when they want to: some cognitive avoidance is inevitable, and it can be a successful and adaptive short-term strategy; and it helps children to feel in control of their thoughts and feelings. Try to summarise for the group by saying something like:

So, it sounds like we all avoid thinking and remembering sometimes, and this is OK. In fact, it can help for a short while. Remember that in the first group, we even practised ways to help us avoid remembering, like listening to music or doing some of your favourite things. You can carry on doing this when you want to if it helps.

But it's very difficult to push the memories away for good - in the end, they'll probably come back later on, so the best thing to do is to face up to them when you're ready. The best way to do this is to deliberately bring the memories up when you want to, and not when the memories want to. The thing that the traumatic memories hate most of all is to be out in the open for a long time. We know that when you deliberately remember and bring the memories out into the open, they lose their power, and just become like normal memories. Next, we're going to show you some ways to bring the memories out into the open so that they're not so scary. Remember that you're the one in charge of your memories now.

Note

Techniques for processing memories – different approaches for different groups. Depending on the children's age, level of maturity, and educational experience, writing is often a most effective means of processing memories. For older, as well as

younger children, drawing is also very helpful. Where the technology is available, recording their experiences onto a mobile phone and then listening to it later can be useful. For children and adults alike, talking about the event with a trusted adult is helpful when done with a good listener. Choose one or more of the following:

Each member of the group is given paper and pencils. Ask them to draw some aspect of their traumatic memory. Remind them that they made drawings of their bad dreams in the first session: here, the aim is to draw something of the intrusive pictures or memories that they have during the day. It should be something which is fairly hard to think about, but which they should be prepared to discuss with the group afterwards.

The aim here is to give children practice in recalling their memories, rather than to experience intense exposure. Have the group split up and allow enough time the children to become absorbed individually in their drawing. The aim is not to produce a skilful piece of art, but for the children to express themselves. The content or colours in the drawing should not be interpreted as having any hidden or symbolic meaning.

The first way of getting the memories out into the open is to draw them, just like before when we made drawings of bad dreams. After we've drawn them, those who want to can talk about their drawings to the rest of the group. Choose something that is upsetting to remember, something that comes back to you unexpectedly or when you don't want it to. It doesn't have to be the very worst thing, but something about your traumatic memory that you want to beat. Don't worry if the drawing isn't very good - the idea is just to get the memory out into the open. Now find somewhere good to draw on your own in the room. And remember - what's the thing to do if you start to get upset when drawing or telling about the memory?

→ ***Elicit*** *coping strategies e.g. Safe place, Calm breathing etc.*

That's right – you can use your Safe place or one of the other Tools in your Toolbox

→ ***Handout*** *paper and pencils and walk around the group*

→ ***Encourage and comment*** *on individual children's drawings*

→ ***Allow*** *time for each child to become absorbed in their drawing.*

OK. We're going to stop now. It doesn't matter if you haven't completely finished. Let's get back in our circle and we'll have a look at all the pictures and those who want to can tell us a bit more.

Who wants to start telling us about what they've drawn?

Note

Retelling: By describing their drawing children are getting practice in retelling their story. Traumatic memories are usually stored in a fragmented, jumbled way which makes them difficult to process. The group leaders encourage the children to put a coherent structure to what they remember and to detail events and feelings. All children should show their picture and tell the group what they drew. Some children will be able to tell the group more. All children can say what it felt like to do the drawing.

What did you draw?

Can you tell us about it?

→ ***Help*** *the child talk about the event if they want to: -*
In sequence *- what was going on before, what happened at the time, what happened after*
In detail *– what was seen, heard, felt and thought….*

→ ***Praise and say*** *how difficult it is*

→ ***Encourage*** *each child to tell their story and mutual support*

What was it like to draw it?

How does it feel to talk about it now? What does your fear thermometer tell you?

What's the thing to do if you start to get upset when drawing or telling about the memory?

> → ***Allow*** enough time during the exercise or give time for relaxation at the end so that fear ratings come down.

Do you think this bit of the memory will come back more or less now?

Could you draw something harder next time?

You're all doing really well. It can be upsetting to talk about something you don't really want to remember. But just like fear went down when you faced your reminders, upset goes down. And drawing and talking about memories is a way of fighting back against them. You've shown that it is possible to remember bad memories in some detail without being overwhelmed. It's not easy but you've brought some bad memories out into the open and shown that you can be in charge of them. Well done everybody.

> *If there's enough time …*
>
> → ***Ask*** the children to draw something constructive – e.g. themselves with new friends, how they want their house to be rebuilt after it has been destroyed

Let's finish our drawing with a nice picture. If your house got destroyed in the war it might be what you'd like it to look like when it's been rebuilt. Or maybe you'd like to draw yourself with the new friends you've made. If you draw like this at home you can also look at the drawings and tap your knees like we did in a previous session. Then you can notice the change. If you tap while looking at a nice drawing you make, the good sensation may spread to your whole body.

TOOL: Writing or Recording

Note

As with drawing, writing can be a means of imaginal exposure directly, and can be used as a basis for talking through the events. In the group setting, there will not be enough time for each child to produce an individual piece of writing, so the aim here is to give advice for writing activities that can be practised later. We think that writing usually is best suited for adolescents. Children should be given guidelines on the best way of writing. Put these guidelines up on the flip chart. The writing should be done as a sequential story. They should write about their innermost thoughts and deepest feelings for writing to have its optimal effect (see Pennebaker's research i.e. Pennebaker, 1997). See also www.childrenandwar.org for a writing manual developed for adolescents

Just like drawing, writing the story of what happened is a really good way to get the memory out into the open. This is something you can do later on. Here's how to do it

It's not like writing something for school - things like spelling and handwriting don't matter. The point is just to get all the details of what happened out into the open. You'll need to write what happened as a story – with a beginning, a middle and an end. And in it should write what you were thinking at the time and how it affected you – your deepest thoughts and feelings.

Note

There are a number of ways this exercise can be adapted to suit local circumstances. Each child can be given a small exercise book with their name on it if they are available.

→ ***Write/show*** on a flipchart the following Guidelines for constructing a story

AND/OR

→ ***Hand out*** individual booklets or leaflets with the instructions included

Here are some ideas you can use to help you put your story in order and to give it detail

- What happened before the traumatic event
- How I first knew something was wrong
- What happened next
- What I did
- How the hurt happened
- How I knew it was over
- What I did afterwards
- What others did afterwards
- The very worst moment
- What I learned from what happened

Choose your own time to do the writing. Try to keep your note-book somewhere it won't get lost or where no-one else will take it. Later, when you're feeling relaxed, re-read it. Do you want to re-write it with more details as they come to you? If you haven't done this already try re-writing it in the first person … use "I" …and as if it were happening right now. Maybe you'd like to read it into a mobile phone and listen to it later. That way you might notice more things you'd like to change

Think about who you want to share your writing with – if anyone. Do you want to keep it as your own private, special record; or, would you like to show it to someone else – maybe a parent, a teacher or some other adult you trust? If you want, you can use it to talk through what happened just like we did with the drawings.

Don't forget – the reason for doing the writing is to get things out in the open. When they are out in the open they will start to lose their power over you. You may not finish your story all at once. And if things are still difficult you may want to carry on writing as a sort of diary to help sort out your thoughts and feelings.

Remember this is your story. It's your writing and it's up to you how you choose to use it and who you want to share it with.

TOOL: Talking

Note

Sometimes, children are able to talk to each other, their parents, or someone else. Commonly they avoid talking to others because they anticipate that talking will be too upsetting for themselves or for the other person. Children and parents can get locked into cycles of not talking. It is important they understand that this sort of avoidance maintains Post Traumatic Stress. The children have already talked a lot about their trauma in the groups. This can be used to begin a discussion encouraging them to talk and express their feelings.

As well as drawing and writing, just talking can help get the bad memories out into the open where they can't hurt you. Over the last few weeks, we've all talked to each other about difficult things and about our experiences during the war. What has it been like to talk?

→ ***Encourage*** *discussion about the advantages and disadvantages of talking. Aim to get them to agree, on balance, it's good to talk.*

→ ***List*** *on the flipchart. These might include:*

Disadvantages -

- it can be upsetting
- people might laugh
- you might upset the other person
- don't know what to say
- don't know the words for the feelings
- might upset mother or father if they know you feel bad
- makes me feel confused
- don't have anyone to talk to
- makes me think I might lose control if I let it out

Advantages

- makes me feel better
- helps me get clear what happened

- find out if others experience the same or similar things
- gets it all out
- someone can help if you tell them
- you can get advice on what to do
- you can help other people
- mother and father won't worry if they know what's wrong
- friends can help if you tell them
- 'a problem shared is a problem halved'

So, we've all been talking over the last few weeks, and although it's been hard at times, in the end it's helped because we learned things to do. Just like drawing and writing, talking helps by getting the memory out into the open. Just like with drawing and writing, there are some things we can do to make talking to someone a good way to beat our traumatic stress. And as a last homework I'd like you to

→ ***Write/Show*** *on the flip chart*

→ ***Discuss/ Direct*** *the group to*

1. Choose someone to talk to who you trust. If you can't think of anyone to talk to in the home environment, you can talk to a teacher at school or another person you trust. It could be another relative, or maybe a special teacher at school. If you have a counsellor, nurse or other person you trust at school, you can go to that person

2. Decide when to talk. You should try to choose a time when they are around and not busy. You should not have to talk if you do not want to. No-one should force you to talk. Don't talk just before bedtime. Let the adult know you want to talk to them. Maybe fix a regular time with parents, set aside for talking

3. Think about what to say. Sometimes we don't know what we want to say until we start talking, and that's OK. You don't have to tell it all at once - go at your own pace and talk as much as you want to.

Note

Looking to the future: Finishing the session on a more positive note to help to balance the feelings of hopelessness and helplessness that can often result from exposure to war. With older children, it is particularly important to allow enough time for this discussion at the end.

With adolescents, this kind of discussion often touches on existential aspects of war and life, with themes of guilt and justice. Younger children, need a more playful approach. You may want them to fantasise about what they wish for the future. You could use some of the techniques from previous sessions e.g. changing drawings or story endings so that they have a positive ending, even if this is fantastical.

Closing Option 1. Adolescents

We're coming to the end of our last session now. Over the last weeks, we've talked a lot about things that have happened in the past. Now we want to spend time thinking about the future

Note

The aim of the final discussion is to foster a realistic sense of hope and continuity; and a feeling that there are things that young people can do to change their situation.

→ ***Encourage*** *a general discussion with open ended questions*

What do you hope for the future?

What do you hope for your families in the future?

What do you hope for the future of your country?

How do you see things in 5 or 10 years' time?

What do you think you can do to get there?

Closing Option 2. Younger children

Note

The aim of the final session with younger children is to help them have fun and imagine what they wish for the future. This can incorporate earlier techniques such as drawing their hopes for the future.

We're coming to the end of our last group now. And you've all worked REALLY hard over the weeks and it's been lovely being with you. You've talked a lot about the things that have happened to you. And even if you got a bit upset sometimes, we hope you found out that it helped in the end. So now you know it's good to talk – to grown-ups and even to each other. And you've drawn some good pictures too… about some of the hard things. But also about nice things you'd like for the future. You've all learned and remembered lots of useful TOOLS to use to help you when things get a bit much. But let's all finish now with a really STRONG THOUGHT for yourself – I DID WELL! Because you all did really well.

So after three let's all shout together – so everybody knows – I DID WELL and afterwards go round everyone giving a fist-bump/high-five.

One…Two…Three… DIDN'T I DO WELL!!!

PART 3: Bereavement

[adapted from Dyregrov 2008]

Background

It is not uncommon for children living in a disaster area to suffer the loss of a close family member or a child. Although this manual focuses on the traumatic after-effects of the disaster, many reactions to loss are similar to those caused by exposure to the disaster itself. This section will help you understand bereaved children and what can be done to help them. At the end of this section, we also have included a short description of a series of group sessions that can be undertaken to help bereaved children.

Principles

Children who live through disasters and violence are often bereaved as a result. This section contains material to help children grieve. First, the introduction provides information on the sorts of ways children respond to death, with the emphasis on children's responses to sudden traumatic death Then ideas, advice, and activities for adults to use to help children are presented. These could be integrated into the parents' group, or adapted for use in schools. Last, ideas and activities for use with children are given. These may be integrated into the last of the children's groups; or you may want to organise a separate group especially for children who have been bereaved.

This section on grief in children is presented separately from the other material in this kit for two main reasons. First, children's experience of death varies considerably in different disasters. In some disasters, children may have little direct experience with death, and much experience with destruction of property and the community. In others, it may be common for most children to have experienced the death of one or more extended family members or friends; and in yet other disasters, children may have witnessed the deaths. Depending on the nature and extent of experience of death that children in your community have, you may choose to use these materials differently, perhaps with selected groups of children.

Second, beliefs about death and the rituals and customs surrounding it also vary from community to community, depending on culture and religion. As will be seen below, these customs play a very important part in children's grieving, so the ways in which children are helped to grieve will differ depending on their cultural background. We have tried to set out basic principles for helping children below, but it is again up to you to decide how best to adapt these for use in your community.

Children's development of understanding of death

By the age of 8 or 9 years - the youngest children in your group - children will generally understand that death is the irreversible end of all physical functioning. Children of this age may still be rather concrete in their thinking and tend to focus on the bodily aspect of dying. They know for example that dead people cannot speak or move, that they do not breathe or eat, and that their heart has stopped beating. They can understand death as both the result of external causes (i.e., violence) and inner processes (i.e., illnesses), and their interest may centre on the physical causes of death and the physical process of decomposition. Although these early school age children begin to understand death as universal and unavoidable, they may find it hard to conceive of death as a possibility for themselves. Some children of this age begin to develop more abstract concepts of death. These may have a "magical" component, for example in assuming that the dead person can still see or hear the living, and working hard to please them as a result. Children of this age are able to comprehend the perspective of others, and can show compassionate and empathic feelings towards friends who have been bereaved. Older children and adolescents develop an additional understanding that death is inevitable for everyone, and will happen to them personally. Their concept of death becomes more abstract, and they may begin to question whether a soul or spirit exists, and if so what may happen to it at death, as well as physical changes that occur. Adolescents may reflect on justice, meaning, and fate; and perhaps on occult phenomena (omens and superstition) as well.

Grief reactions in children

There is no right or wrong way for children to react to death. Children may react in a variety of ways. Common immediate reactions include: shock and disbelief, dismay and protest, apathy and being stunned, and sometimes

continuing with usual activities. As grieving proceeds, children often show some of the following: anxiety, vivid memories, sleep difficulties, sadness and longing, anger, guilt, school problems, and physical complaints. Other reactions may occur. Children might show regressive behaviour, social isolation, personality changes, pessimism about the future, or a preoccupation with cause and meaning. This variety of reactions makes children's grief seem confusing for adults, and it can be difficult to know how to help. Some of these reactions are described in more detail below.

Immediate reactions

Shock and disbelief ("It can't be true", "I don't believe you") are common in older children especially, and parents are often surprised that children do not react more strongly. However, it does not mean that something is wrong if a child reacts in this way: this sort of denial is a necessary and helpful protection mechanism that prevents children from becoming emotionally overwhelmed. Other children may react more strongly and can become quite inconsolable, crying for several days after the death. Yet other children might just carry on as if nothing had happened ("Can I go out to play now?"), seeming as if they are on autopilot. Again, this sort of reaction may serve a protective function, allowing children to continue with ordinary and well-known activities at a time when the world seems chaotic and unsafe.

Later reactions

Fear and anxiety is common in children after they have been bereaved. Children who have lost a close family member often fear that the remaining parent will also die ("If it happened to father, it could happen to mother, too") and older children often think through the consequences of this ("who will take care of me if you die?"). The fear that someone else may die tends to be more common than the fear that they themselves will die, although some children do develop a fear of dying themselves. This can result in separation difficulties or clinginess, even in older children, for example, in a fear of sleeping alone, or a refusal to stay alone at home.

Sleep difficulties are common, and the problem can be one of getting off to sleep, or waking during the night. This is more likely if the word "sleep" has

been used as a way to describe death. Sometimes, children are afraid to go to sleep for fear that they will not wake up.

Sadness and longing appear in different ways. Children may cry frequently, or become withdrawn and apathetic. Some children try to hide their sadness so as not to further upset their parents. Longing for the dead person can show itself when children are preoccupied with memories of them, when they feel the dead person's presence, or when they identify with the dead person. Children might seek out places they used to visit with the dead person, or engage in the same activities they used to do together to make them feel closer to the dead person. Children may sometimes want to look at pictures of the dead person, ask to hear letters read out, or ask to hear stories about the dead person. This can be distressing for adults, but is a normal way for children to come to terms with the loss of a loved one. On occasion, children may feel that they have seen the dead person, or heard their voice, for example at night-time. This is quite normal in adults and children, but can be very frightening if children are not prepared for it.

Anger is also common in grieving children. It tends to show itself more often in boys, and might take the form of aggression or acting out or temper tantrums. Children may feel angry at death itself for taking the person away from them, or at God or Allah for letting it happen, or at adults for not preventing it (or because adults have excluded the child from their grief), or at themselves for not having done more, or at the dead person for deserting the child. Angry feelings may be connected to guilty feelings. Guilt can arise when children feel that they did not do enough to prevent the death, or even that they may have caused or contributed to the death. Guilt may derive from the kind of relationship that the child had with the dead person, for example when the child regrets things that were said or done when the person was still alive. Grief can lead to school problems, particularly in attention and concentration. Thoughts and memories of what has happened can interfere with schoolwork, and children who are grieving tend to be slower in their thinking and may lack energy or initiative. Physical complaints may be present, and can include headaches, stomach-aches, soreness or aching, and fatigue.

The sorts of reactions listed above are far from comprehensive, but do illustrate how varied children's reactions may be after death. Four broad stages in the grief process have been described. The first is often a relatively short phase of shock, denial, or disbelief. The second is the phase of protest, where children may be agitated and restless, and might cry out or search for the dead person. The third phase has been characterised as one of despair, accompanied by sadness and longing, and perhaps anger and guilt as above. The fourth phase is one of acceptance and resolution. It can be useful for helpers to conceptualise the grief process as a series of stages in this way. However, the progression through these stages is not smooth, but is rather a moving back and forward between stages as the grief is worked through. Remember that there is no right or wrong way for children (and adults) to grieve.

The range of "normal" grief reactions is very broad, but in some children grieving can become complicated. That is, there may be no grief reaction; or it may be delayed, prolonged, or distorted. All children need support in grieving, but those children who show complicated grief reactions are especially in need of help. There is evidence than when children are unable to grieve at the time of death, they are more likely to be affected throughout their lives in all sorts of subtle ways. It is not possible to predict which children will show complicated grief reactions, but there are some types of death, which are likely to make grieving more difficult. Unfortunately, the sorts of death that are likely to result in a complicated grief reaction are all too common during disasters.

Children's experience of death during disasters

Any kind of death is distressing for children. However, children's experience of death in disaster generally differs from more "normal deaths" in ways which are likely to make it even harder for them to grieve well. Children's experience of death during disasters can take many forms, but in general, three main factors make it harder for children. First, in contrast to "usual deaths", death is more likely to be violent and witnessed by the child. Second, many people are killed during disasters. Third, the threat of death of others can continue to be objectively high after the child has been bereaved i.e., because of new

earthquakes. Forth, it is not uncommon for the body not to be found, and the lost one is just missing.

Where death is sudden and violent, it is called a traumatic death. Traumatic death is especially hard for children, particularly if they witness the death. Then, many of the sorts of normal grief reactions described above can be very intense, and they will often be combined with the sorts of posttraumatic phenomena that were described previously. So, for example, children may have vivid intrusive images or memories of the death, or they may suffer from nightmares. Where children have witnessed a traumatic death, they are even more likely to be anxious, jumpy, and hypervigilant for danger. Where many people have been killed, or if the threat of being killed remains high because of the risk for a new disaster, then children may understandably fear that others - parents in particular - may die, and the sorts of anxiety or separation problems described above are likely to be more intense. The scale of death during disasters means that children's "safety assumptions" about the world are quickly undermined. For older children, this can often mean a good deal of questioning issues to do with justice, fate, and meaning.

It is important to remember that "normal" deaths continue to occur in the period following disasters. Grandparents or great uncles and aunts may die naturally. These sorts of death should not be passed over by adults in the expectation that they should somehow be "easier" for children. A death like this will serve as a reminder for children of the universality of death, and space should be made for the child to grieve specifically for that person amidst all the confusion of disaster.

Helping children to grieve

Death is one of the hardest experiences for children to deal with, and one of the hardest for adults to help with. In the sections below, some ideas are given for ways in which adults can help, and activities that may be done with children in a group or in school. Adapt them in the most appropriate way for use in your community. Religious rituals and practises are helpful after death: we talk about these below in general terms, but use the guidelines in whichever way is most suitable for your community.

It is our experience that after traumatic deaths of the kind commonly experienced during disaster, the grieving process is interrupted by post traumatic symptoms. For example, a child who has seen his mother being killed may be unable to remember the good times he spent with her and to begin grieving because whenever he thinks about her, distressing intrusive images of her death come to mind. It is hard to help such a child to grieve well without first addressing and helping with the posttraumatic stress phenomena. We recommend that these be addressed first. In using some of the ideas below, you will therefore need to be able to adapt and use the activities in the group sessions described previously and combine them with those below to help children effectively.

How adults can help children grieve

There is a great deal that can be done to help children who have been bereaved in disasters. Children need time to work through both the emotional and cognitive aspects of bereavement, and activities should be geared so that both of these dimensions are stimulated. Dyregrov (2008) summarises some guidelines for adults who are helping children: these are useful for parents and teachers alike.

Open and honest communication

- give age-appropriate explanations
- reduce confusion
- don't give abstract explanations
- don't explain death as "a voyage" or "sleep"

Give time for cognitive mastery

- allow questions and conversations
- accept short conversations
- look at albums and photographs

- let children visit the grave
- accept children's play

Make the loss real

- let the child participate in rituals (seeing the dead, funeral)
- do not hide your own feelings
- keep reminders of the dead person present

Stimulate emotional coping

- work for continuity in home, school, or play group
- avoid unnecessary separations
- talk with children about their anxiety about something happening to their parents or themselves
- talk with children about eventual guilt feelings

Some of these guidelines are explained in more detail below.

Participation in the funeral

If possible, children should be allowed to participate in funerals or memorial services. Sometimes, we think that it will be too upsetting for children to attend funerals, and we try to protect children by excluding them from the ceremony. But children, just as much as adults, need to gain a concrete basis for their grief. The funeral service, as well as being a celebration of someone's life, is a public way of making the death real. When children participate in the ceremony, they can have a concrete basis for their grief. That is, they will see the dead body, and see that it is burned or buried.

But children should be prepared properly for the funeral. This means explaining what will happen during the ceremony, and if the body is to be viewed, describing beforehand what it is likely to look like. Children should

also be prepared for adults' strong reactions; and can be told, without frightening them, that they will probably feel sad or anxious too. Depending on the sorts of services held in your community, children can participate in the funeral itself. This might mean reading or saying prayers, or helping physically with the burial. It might be more personal, for example in touching the body, or placing something personal in the casket or on the coffin. It is important that afterwards children are given the opportunity to talk through what has happened and to ask questions.

Often during disasters, the normal rituals and ceremonies surrounding death are not possible. Sometimes, because of the fear for disasters, bodies may be disposed of hurriedly with little ceremony, or they may be buried in the wrong place; and sometimes the body may not be recovered. This will almost certainly make grieving harder for children. It will therefore be useful if you discuss with other group leaders or in your parents group, alternatives that could be used. This might mean for example having some smaller kind of ceremony at home, perhaps with a religious or community leader visiting the home.

Communicating with children

When we communicate with children openly and truthfully, giving them concrete and direct information, then confusion and fantasy on the part of the child can be minimised. It is best to avoid metaphor or abstract concepts (saying for example, that the dead person is asleep): this can make things more confusing for the child.

Parents may be reluctant to talk for fear of upsetting children further, or because they think that they may break down in front of the child. In a similar vein, parents sometimes hide their feelings of sadness from their children, not wanting their children to see them crying and grieving. If parents are continually overwhelmed and cannot talk to their children about the dead person without very strong emotions breaking through, then it may be best for the child to have a trusted adult outside the family with whom they can talk. However, when possible, parents should be available for their children. If they do not talk, and hide their grief, then the child gets the message that it is

wrong to talk about the dead person, wrong to cry, and that strong emotions are intolerable. They may also feel that the parents do not care about or remember the dead person if they are never able to speak about them.

Children need to be given time and permission to talk to parents and others about the dead person. Time needs to be given for cognitive mastery, that is, for children to develop an understanding of what has happened. Children will do this by asking questions, depending on their age. Such questions are often penetrating and can be painful for adults to hear and difficult to answer. Questions may be to do with the physical or spiritual aspects of death, or personal questions about the dead person. Carers need to be prepared for such questions. Answers should be truthful and to the point. If you don't know the answer, it is best to say so. If a question is very painful, you might say that you need to think about it, and you will talk to them later, but if you say this, make sure you follow up.

Adults should also be prepared for conversations of this sort to be rather short. This can be shocking for adults: children may ask very penetrating questions, and then go out to play. They may come back with a similar question later. Remember that children need more time than adults to grasp what has happened: their understanding develops in a gradual, step by step fashion, and they may "tumble around" with words and ideas. Children have a shorter "sadness span" than adults and are less tolerant of strong emotions.

Some children may refuse to talk about the death or the dead person. This may be the child's way of protecting himself, and care should be taken not to force children, or push them too quickly. As with the advice in the Avoidance session for children, the aim for adults is to provide the sort of emotional environment where children are able to explore their feelings, rather than pushing them into talking of things that they are not ready for. Here, using means other than talking are useful: all children need to have some concrete means of working through their grief, and some of these are described below.

Concrete expressions of grief

Concrete means of expressing grief are a way of making the unreal real. This can be stimulated by keeping memories of the dead person present and visible, as part of the child's daily surroundings. Gradually, the parents and child can remove those things that it will be unnatural to keep. It helps children to remember the dead person if they have objects or memento's which remind them of times they spent together. You may have photographs or pictures of the dead person, and children should be able and encouraged to look at these, perhaps with a parent so that they can talk and ask questions if they want to. Some children like to have special objects - clothing, jewellery, tools - which they can keep in their own special place and look at privately when they want to remember the dead person. Some disasters leave little in the form of memento's and adults may have to help the children get hold of for example pictures from distant relatives, or help them search the rubble or remnants of their building for such objects. Visiting the graveside is often a way for children to come to terms with death. Depending on their age, they may want to do this with family or friends, or alone. Other children may want to make drawings of the dead person, or of gravestones or religious sites. These are not morbid preoccupations, but show the ways in which the child is thinking about and coming to terms with their loss. Parents can encourage this, and sometimes it can be a means to begin talking with children about what has happened.

Emotional coping

As well as developing an age appropriate understanding of what has happened, through talking, drawing, playing, or visiting the gravesite as above, children also need to find ways to cope emotionally. Much of the advice given in the previous sessions applies here, too. Children need to feel that they can talk to their parents when necessary, and that they can continue to play and enjoy themselves at times. It is important that a normal routine as possible is re-established: this gives children a sense of security and safety in these very difficult times. Children who are anxious or fearful after they have been bereaved may benefit from using the relaxation techniques described in the second group session.

Parents' own needs

Perhaps the most important thing that parents can do for their children after a death in the family is to look after their own needs. Parents should be aware of the sorts of responses that they and their children may show after someone had died. Some of the advice in section 3 of the parents' session applies here. Parents need to give themselves time to grieve if they are to help their children. This can be helped by observing normal mourning periods and anniversaries as far as is possible in the aftermath of disaster. Parents may also find it helpful to seek out peer support: that is, to recognise that at times of crisis, they cannot take on too much, and it is OK to ask for help from others.

Activities for children

Grief can be a slow process, and much of the "grief work" that children do will be within an emotionally supportive environment at home. The following are suggestions of activities for children to do which can help the grief process: they might be suggested to parents as ways of helping their children, or incorporated into one of the children's sessions above, or used as part of a grief group for bereaved children, or used at school by teachers who know the children well.

Letter writing

Children may sometimes find it odd at first to write a letter to a dead person, but it can often be a great help. Children might be asked what they wish they would have said to the person the last time they saw them, if they knew that that was going to be the last time. What would they say to the person now, if they could speak to them? Does the child feel that he or she has said goodbye to the person who died? Have they told them all they wanted to tell them? Children can be asked to write all this down, in a letter to the dead person. Sometimes, children will then use the letter later to show to a trusted adult as a basis for talking about their grief. More often, children will want to keep the letter themselves as something private that they can refer to and re-read whenever they want to think about or feel close to the person who has died.

Talking to the dead person

Children are sometimes reluctant to say that they hold internal dialogues with the person who has died, but this is very common, and a useful way of working through grief. Children might just want to tell the dead person what is happening now in their life, or to ask them for advice. Children may be embarrassed that they do this, but it should be encouraged for it is a way of remembering the dead person while at the same time letting go. It will gradually diminish naturally over time.

Journals and diaries

As with letter writing, and as in the writing tasks in the Avoidance group for children, keeping a diary or journal can also be an effective way of working through all the grief reactions. Children might keep a personal diary or log of events and feelings. Sometimes, this is a way of keeping an internal dialogue going; sometimes it is way of structuring and making sense of the bewildering array of feelings that accompany bereavement. Again, diaries might be used as a basis for talking if the child wants to; or it may be kept as a private and special way for the child to remember.

Prayer

Prayer and other religious practices can be very useful for children. When the child is not from a religious background, then a time of bereavement is not the best time to introduce such concepts. Still, most people, religious or not, take comfort from some kind of prayer or meditation at a time of death. Children are the same, and depending on their age and developmental level, they may talk to God or Allah, asking them to take care of the dead person, and of those still living.

Rituals and mementos

Children need to have some concrete means of expressing their grief. It can be very helpful for them to participate in the funeral or burial. Later, it can be helpful for children to visit the grave. This might be done with other family members, but older children often like to go alone so that they can remember and think about the dead person. Often, this is kept quiet: children feel that parents will worry if they know they are going alone to the graveyard or burial

site; or children may like to keep it as a special private way to "be with" the dead person when they want to. For some children, it is harder to go because they may fear becoming too upset, or because the graveyard is frightening. It should be encouraged, and children can be helped to find ways to visit the grave regularly, perhaps with family or friends.

Photos or pictures of the dead person are helpful ways for children to remember. Sometimes, children might be encouraged to look at them with parents, or be given permission to look at them when they want to. Otherwise, children might like to keep special objects which belonged to the dead person, or which remind them of the dead person. As mentioned above, following disasters children may need adult help in recovering such mementos.

Group activities

In school or in a group, it can be helpful to speak about death and dying in general terms, as well as or instead of doing focussed grief work with children. For example, it may be helpful to teach about death in an open and straightforward way. With younger children, this might include discussions about what happens to our bodies when we die. With older children, they welcome the opportunity to discuss in groups the spiritual, religious, or existential aspects of dying. Again, where abstract discussion is hard for children, classroom or group activities might focus on the rituals or more concrete aspects of dying, for example thinking about why we hold funerals, and what happens at them. Children's coping may be stimulated by having a group or classroom brainstorming session on how children would help other children who have been bereaved.

Manual for bereaved children.

Following feedback from the field following war and disaster, the Children and War Foundation has developed a grief manual (Dyregrov, Yule, Straume, & Kraus, 2011) that can be used for helping children in groups. The manual has now been tested and found to help children presenting with traumatic grief. See www.childrenandwar.org for details of training opportunities.

PART 4: Parent groups

Introduction

One of the most crucial factors in helping children to cope with the stresses of disaster is their home environment. In parallel with the sessions for children in the previous sections, this section contains material to run a group for parents so that they are better placed to help their children. The aims of this parent session are:

- to normalise children's reactions
- to improve the recovery environment of the child
- to give advice to parents on self-help strategies
- to inform parents about the techniques children learn in the children's group
- to link parents with specialist or school's services for children

It is intended that the session for parents be carried out by the same facilitators who run the children's groups, so that there is an established link between parents and families. Preferably, the parents group is held before the children's sessions, so that parents can begin to help children immediately after the first children's group. The guidelines here should be used flexibly in conjunction with the guidelines for the children's groups, where the specifics of particular techniques are detailed. Adapt and use the sections here as is most appropriate for your group of parents.

1. Normalising children's reactions

It is important for parents to say how their children have been affected. In a group context, this means that they will hear that they are not the only parents whose children have been affected, they will have a framework to understand

their children's reactions, they will begin to make the links between adult and child reactions, and can start to develop ways to help children.

Introduce the purpose of the group, and then ask parents what changes or reactions or problems children are having because of the disaster. Write these down on a flipchart for everyone to see. Encourage parents to share similar experiences with their children. These might include:

- nervousness
- jumpiness, especially at loud noises
- fears
- worries
- anxiety
- frightened to go outside (or inside when their building collapsed)
- withdrawal
- depression
- crying easily
- getting upset over minor things
- not wanting to play
- irritable, quick to anger
- temper tantrums
- restless
- problems concentrating
- problems sleeping
- nightmares
- playing disaster games
- bereavement reactions

- fear of death of other family members
- lost interest in the things they do
- moodiness or rapid mood changes

Parents need to know that these are common reactions. They are normal in the sense that most children will experience some of them at some point, although most children will recover. Every child responds differently (according to their age, gender, background, temperament etc), and there are no right or wrong reactions. It can sometimes be helpful for parents to discuss positive changes that they have seen, as well: some parents say that children are more mature, more responsible, value life more, have become more caring and help others more, for example.

Parents also need to know that these are understandable reactions, in terms of posttraumatic stress. Adults are very good at noticing overt changes in behaviour, but it is much harder for them to know about children's internal distress. In this case, it is helpful for them to make the links between adult and child distress. Ask parents if they have distressing memories themselves, and whether they think the same might be true of children. Ask directly if their children have told them about distressing memories. Have they noticed any behaviour that would suggest that they have distressing memories?

Introduce the idea of posttraumatic stress reactions. Children do commonly have distressing intrusive memories after exposure to disasters. These may be in the form of nightmares, waking intrusive pictures or thoughts, or be revealed in repetitive play. Use case examples of your own, or ask for examples from the group. As with adults, the natural reaction of children is to try to push these thoughts away, or to avoid reminders of the event that can trigger them. Again, make this relevant to parents' real-life concerns by asking the group for examples of avoidance in themselves or in their children. Behavioural and cognitive avoidance may provide temporary relief, but in the long term, prevents the traumatic memories from being processed adequately, so it is likely that they will return in the future. Illustrate the cycle between intrusive symptoms and avoidance, and explain how avoidance maintains

intrusion. Increased arousal often accompanies these symptoms. The aim of educating parents about posttraumatic stress is for them to have a framework to understand the changes seen in their children, and to know that such changes are normal. Every care must be taken to ensure that parents do not feel that by talking about posttraumatic stress, their children (or themselves) are being labelled as sick, mad, ill, or cases.

Be sure to link this posttraumatic framework to the sorts of problems that parents listed before, so that it is relevant to parents. Be clear about how posttraumatic stress can underlie many of the difficulties that parents have noticed in children. Posttraumatic stress reactions are by no means the sole reaction in children who have survived disasters, but are common and distressing, and therefore deserving of attention. Understanding children's reactions like this is useful because it suggests ways of helping them.

2. What can parents do to help children?

Ask parents how they have helped children with their difficulties. What worked? What are the most difficult problems to deal with? What are the obstacles to helping children? This is a good point to talk about general ways to help children. Cover the points below, either by general group discussion, by brainstorming ideas and using the flipchart, by splitting up into smaller groups, or by some of the exercises outlined at the end.

Talking and listening

During and after disasters, children can feel vulnerable, frightened, and insecure. Families can be split up and normal daily life seems impossible. Children can become confused as well as scared at what is happening around them. They often become afraid of their own reactions, fearing that they are going crazy or losing control. They may worry about the future, and about their own safety as well as that of those closest to them.

Some parents think that by not talking to children about the terrible events of the disasters, children will forget them. This is a mistake. Although children, like adults, need to have time to switch off from thinking about the event, they

will not be able to forget what has happened to them. Talking about what has happened in the past and about their worries for the future with someone they trust will help children to make sense of what is happening around them and their memories will become less distressing.

Children need to make sense of what is happening around them, at a level appropriate to their development and age, if they are to feel safe and secure. They need time to talk about what has happened to them, and to express their feelings associated with bad memories. This requires parents to be ready to listen to children. Sensitive listening means finding the right time to talk, and leaving enough time for children to talk and express difficult things fully. It means showing you are attending to what the child is saying, and that you are interested, by making encouraging comments and questions, and by your body language and tone of voice. It means accepting all the child's feelings as natural and normal and OK in this situation, even if these are difficult for a parent to hear.

Sometimes, when the pain of a child's story is too much to bear, adults' natural response is to avoid the pain and stop the child's distress by saying something like, "don't cry now", or "we don't need to talk about that". This will give the child the message that the adult does not want to listen, and that there are unspeakable things that cannot be mentioned. Inadvertently, by trying to ease children's suffering, we can instead encourage them to bottle up their painful memories and distressing emotions. Although it is very hard for parents to listen to children when they are upset, it is important for the child to be able to express his or her feelings fully. This means staying with the child when he is upset, providing comfort and support, and encouraging him to continue talking at his own pace. It is important not to stop when the child is upset. Stay with him and comfort him until he has told his story and feels a little better; then give plenty of praise for having the bravery to talk.

Sometimes, the problem is of a different sort, and children are unable or refuse to speak to their parents about what is troubling them. Children cannot be forced to talk, but parents can show them and tell them that they are ready to listen and help whenever the child wants to. Children may respond to

physical comfort and massage (see below), even if they are not ready to communicate. For other children, ways other than words can help them to communicate what is troubling them, so that drawing or playing with mother or father is sometimes helpful. Here, drawing would be used as means of getting started on communicating, or as a basis for talking about things, and never to be interpreted as having some hidden or symbolic meaning.

Even young children ask questions about the disaster, and these can reveal their confusion or worries. Children might ask what has happened to someone who has died. They might ask about where their fathers or brothers are if they are missing. They might ask when things will be back to normal. It can be hard to know how to give advice to children, or how to answer questions which are difficult, even for adults. It is important to be honest with children, and to answer in a way that they can understand. Older children may be concerned with the origin of the disaster, and it is important that they are able to discuss these issues of cause and meaning with parents at home.

Group Activities

To explore some of the issues above, the following activities could be used as small group exercises, or whole group discussions.

The Value of Talking

Why is it important for children to have someone they can talk to? Is it better for parents to shield children, or to discuss things with them? What are the disadvantages in trying to create a home environment where children feel safe to express their painful emotions?

Advantages might include:

- gets things out in the open
- parents understand the child better
- relieves tension for parents and child
- makes the child feel better

- talking makes it easier to forget
- child can start to distance self from painful memories
- puts their fear in perspective
- means they don't bottle everything up
- we can help them if we listen to their problems
- child can get a different perspective on things
- child can hear his feelings are normal and he's not going crazy

Make sure the distinction is made between creating an environment where children feel able to express themselves with someone they trust when they need to; and forcing children to talk about difficult things.

This exercise can also be done with parents themselves as the focus. Do they find it helpful to talk? Who do they confide in? How do they feel after they have talked with a friend or partner? What does the friend or partner do to provide emotional support?

Blocks to Listening

Why is it hard to talk with children? Why are some children withdrawn and unable to confide even in their parents? Why do some parents find it hard to hear what their child is saying?

Children may not want to talk because:

- it is too painful
- they don't have the words
- feelings and emotions are contradictory
- they feel guilt and confusion
- they are afraid of not being heard
- they are afraid of losing control

- they don't feel the need to talk
- they talk to others (adolescents talking to friends, i.e.)

Parents may not want to listen because

- it is too painful to hear their child's distress
- they fear the child will get more upset
- they fear they will get upset themselves
- don't know what to say to child
- reminds them of their own traumatic experiences
- it's too exhausting
- don't have the time
- believe it can be harmful for children to talk like this

Often during disasters, parents and children will have lived through the same traumatic experiences. It is very common for them not to want to talk to the other, for fear of upsetting them. This applies as much to children as adults. Children and parents often get locked into cycles of not talking to each other, believing that they are protecting the other by not talking. Parents are right to be concerned about how they would react, and must look after themselves if they are to be available to support their children.

When a child is silent, what should we do? Ideas might include:

- respect their silence
- be patient, look for appropriate occasions
- set aside a special time each day: the child will use it when he is ready
- don't push for more than the child wants to tell
- find ways other than talking to communicate, like drawing and playing

Being a good listener

What are the characteristics of a good listener? Parents can split into small groups and think about someone they confide in: what makes that person a supportive listener?

They might be:

- sympathetic
- understanding
- warm and caring
- take things seriously
- don't belittle problems
- offer advice
- just let you talk, don't say everything will be OK

How is it different when listening to children?

- need more time to express selves
- need help finding ways or words to express themselves
- need to have things clarified for them
- often return with the same questions or worries
- remember idiosyncratic things

How can we show we're listening to children?

- tone of voice
- facial expression
- accepting feelings ("that must have been very scary", "these are very sad things you're telling me")
- asking questions
- set aside a special time each day for talking or playing or being together

In pairs, parents might role-play talking to their own or someone else's child about a difficult experience. Or this can be done in threes, with the third person as an observer to notice what helped in communicating and what seemed to hinder it.

Daily routines and play

After and during very stressful events, normal life can seem chaotic. To develop children's sense of security, safety, and stability, it is important to re-establish as much as possible a normal daily routine. This might include getting children back into school, or regularly attending clubs or activities. It might mean regular family meal times, and set bedtime routines. When normal daily routines are kept to as much as possible, the child's world becomes more predictable and comprehensible.

Children also need time to play and do enjoyable activities, even in times of disaster. Play is part of normal development. At times of disaster, it can also help to distract and relax children. For some children, play can be a means of expressing difficult memories. In all the horrors of disaster, children sometimes need to be told by parents that it is OK to play and feel good and happy. For older children, they need time to spend with their peers: this too is part of normal adolescent development and should be encouraged. Sometimes, older children become engaged in relief or community work. This can have a significant impact because it counters feelings of helplessness, and promotes active coping and allows children to achieve something and begin looking to the future. Sometimes, older children who participate in these sorts of activities come to identify with helpers, seeking peaceful ways to resolve conflict in the future.

Have the group talk about what sorts of things they would like to see their children doing. This might include reading, drawing, playing or listening to music, playing sports, getting involved in youth clubs, participating in community or rebuilding projects. How can parents encourage this? Can parents get together to set up activity groups for children? Can parents from

the same neighbourhood agree to share supervision of groups of children? Where there is a shortage of equipment or supplies, how could parents organise to obtain them or raise money for them?

Touch and physical comfort

Diverse cultures vary greatly in the amount of physical contact that is expected or permissible between adults and children. Equally, individuals vary in the extent that they like to give or receive physical comfort. Touch and massage may have beneficial effects for children, reducing tension, anxiety, and depression. We know that after natural disasters, parents tend to offer less physical comfort to their children. When this was increased, it had a positive effect on children's well-being. Depending on the traditions in your community, discuss the issue of touch and massage with the group, and encourage parents in the physical comfort of their children.

3. What can parents do to help themselves?

For parents to be able to care for their children effectively, it is important for them to look after their own emotional needs. In the sections above, parents will have already made the links between children's and adults' distress, and recognise that adults and children may react in somewhat similar ways. In that case, all the activities that have been discussed as ways of helping children will also be useful for adults. So, just like children, parents need someone to talk to and confide in; and need to be able to get involved in pleasurable activities. The specific techniques in the sections for children are also applicable to parents: imagery changing techniques, relaxation, and gradual exposure. Here, get some recognition and acknowledgement from the group that parents must attend to their own needs if they are to help their children, and ask the whole group for examples of ways they have found to help them cope better. These might include some of the following:

- finding ways to relax
- social support, meeting friends
- doing enjoyable things
- humour, joking, having fun

- getting involved in rebuilding activities
- helping others in a practical way
- being active, planning things to do with family or friends
- thinking about the future
- having a “holiday” from thinking about the disaster, switching off
- accepting uncertainty
- prayer and ritual

If appropriate, this may be the time for parents to consider setting up a support group. This does not have to be formal in any way, but may just be a group of parents agreeing to meet regularly each week for “time off”. Or it may be organised, with parents mobilising to find ways to liaise with school or with relief organisations on behalf of children. As with older children, this kind of active structured activity can combat feelings of helplessness, as well as promote social support networks and have a real practical impact on life in your community.

4. The children’s groups

Parents need to know what will happen in the children’s groups so that they can help children to carry out the skills they have learned afterwards at home as necessary. The skills as taught to children in these groups are all adaptable for use with adults, so parents can benefit directly from learning some of these skills. At the end of this section, parents should be able to:

- help children carry out safe place, imagery, and dual attention tasks
- help children in dream restructuring and drawing positive dream endings
- help children to carry out relaxation techniques
- set up regular routines, including bedtime routines, and other sleep hygiene techniques
- carry out activity scheduling with children

- help children to plan and carry out graded exposure tasks
- help and listen to children as they write, draw, or talk about their experiences

From your knowledge of the content of the five group meetings, introduce the group activities to parents. Go through each of the activities, with parents practising them wherever possible: if parents have direct experience of using the techniques, they will be in a better position to help their children, and may also find that they use the techniques themselves as self help strategies. Pay attention to activities that will ask children to do homework in each of the groups: prepare parents so that they are ready to help children practise and complete homework jobs. The following are guidelines as to what might be covered in this section for parents, but adapt and use it in conjunction with the detailed notes in the children's section.

Intrusion session

Recap on intrusive symptoms if necessary: make sure that parents accept that these may occur in children and know what they are. If appropriate, start by asking what strategies adults use themselves to deal with intrusive imagery, and whether any of these could be adapted for use with children. Introduce the imagery techniques as they are taught with children. If there is time, parents should practise safe place techniques, or image changing techniques (as in the section for children). It is useful for parents to have some direct experience of these techniques themselves if they are to be able to help children do them later; and these techniques can be carried out by parents in much the same way as in the children's groups. Introduce the dual attention tasks. Again, get the group to practice these in pairs, using knee tapping as in the children's groups. Care should be taken here, for these can be powerful techniques. Prepare the parents by telling them the sorts of changes that may occur. Introduce the dream restructuring work as in the children's section; and combine this with imagery work above.

Last, introduce distraction techniques. It is likely that parents will have come up with this earlier as a means of self-help. Go around the group and ask

parents to say what things they do to distract themselves. Make a list on the flip chart. Say that this can also be helpful for children, and compile a similar list for children. Make the point that although it is necessary for children and adults alike to face up to traumatic memories at some time, it is important in the first place to regain a sense of control over them. This means developing ways to switch off from thinking about them by doing pleasurable activities.

Arousal session

Tell parents that the second section with their children is to do with physiological over arousal. Hyperarousal is important because it is strongly associated with intrusive symptoms, and may maintain them; and because it has a direct adverse impact on children's life, for example having concentration problems at school, and difficulties sleeping at night.

Start by making the links between adults' and children's distress. Ask the group for their own experiences of over-arousal. Are parents themselves anxious and jumpy? Do they have problems concentrating on one thing for any length of time? Ask how parents deal with this themselves. What do they do to relax? Reinforce and develop these as appropriate. Perhaps write parents' relaxation strategies up on the flipchart, and encourage parents to help each other with advice about what has worked for them.

Building on what parents have said they find useful themselves, introduce the relaxation techniques taught to children. Have the whole group practice deep muscle relaxation, as in the children's section. Then practice breath control relaxation. Say how this can be combined with the imagery techniques they have just practised.

Parents should be ready to help children practise relaxation, and encourage them to do so regularly, at least once a day, usually in the evening. It will be helpful if parents can monitor children's relaxation practice. A regular relaxation time might be included in the usual family routines, and some parents may want to practice regularly with their children.

The other two components of the arousal session with children are activity scheduling and bedtime routines. It may be that parents have mentioned something similar to activity scheduling already, either as a means of self-help, or a way of helping children. Parents will know that children feel less distress when they are happily occupied, but the difficulty is often in getting children to re-engage in their pre-disaster normal activities as much as possible. Introduce activity scheduling with an example of a diary. Tell parents that a good way for them to help children re-engage is to help children plan out what they are going to do, in achievable steps, and to give them plenty of verbal reinforcement when they do it. For some groups of parents, this may simply mean stressing the importance of re-establishing as normal a daily routine as possible, and giving children permission to do the things they enjoy. For others, it will mean considerable amounts of help for their child in planning activities, and support and monitoring in carrying them out. At this stage, parents may want to plan teaming out to plan and carry out activities and games for children in their neighbourhood.

Last, parents need to know that sleep hygiene techniques, particularly a regular routine before bedtime, are helpful in combating the effects of over arousal on sleep patterns. Refer to the list of sleep hygiene techniques at the end of the notes for the children's arousal group, and talk through them with the parents.

Avoidance session

Avoidance symptoms are tackled in the third group with children. Because the techniques are aimed at helping children to confront traumatic memories and reminders, they will usually need the help of parents in carrying out what they have learned. Let parents know that the avoidance techniques are carried out in the final section with children so that they have had time to practise the techniques learned in previous sessions before confronting traumatic memories.

Before teaching parents about graded exposure, it is important that there is agreement in the group about the need for children at some stage to face up to what has happened to them. Acknowledge that avoidance can be adaptive

in the short term, but that in the long run, it is not going to be successful. Refer back to the introduction and the schematic diagram of the relationship between avoidance and intrusion if necessary. Ask for examples from the group as to how anyone has coped with any simple phobias in the past to illustrate the idea of exposure as a means of overcoming fear. Emphasise that avoidance is the natural thing to do, but that it serves only to maintain fear and intrusive memories.

Behavioural avoidance

This section for parents will need to cover the same material as in the children's section: construction of fear hierarchies, self-monitoring of fear reactions, coping self-talk, specific steps in carrying out exposure, and the difference between maladaptive and functional avoidance.

Introduce the idea of graded hierarchies using one of the parents'own examples of overcoming a phobia; or with an example of your own. The essential point for parents to understand is that when fear stimuli are graded well, each exposure step for children is achievable, in conjunction with the relaxation and other techniques they learned earlier. Proper grading of traumatic reminders is therefore necessary if children are to progressively reduce their avoidance; and parents will need to help children construct fear hierarchies.

As in the children's groups, ask parents to list and then grade traumatic reminders. Remember that grading will be subjective for each child. The role of parents is to assist children in breaking down each exposure into manageable steps. Show an example of a fear hierarchy on the flip chart. Ask for ways it might be modified or improved depending on the child.

If parents themselves do not raise it, raise the issue of functional and traumatic avoidance. The intention is not for children to expose themselves to genuinely dangerous situations, and parents must help children to plan exposure homework if this is to be prevented.

Fear monitoring and positive self-talk may be introduced in much the same way as it was in the children's groups. The point of fear monitoring is to ensure that fear reduction has occurred at each step, and that the child stays in the situation long enough for this to occur. Parents can practise with children anchoring points on the fear thermometer with emotional words, and practise using the thermometer by rating various imagined scenarios. Similarly, parents can help children in rehearsing positive self-talk in preparation to carrying out exposure. If possible, link self-talk in children to that in adults by asking parents what goes through their mind when they are in a fearful situation, and how they deal with it.

The steps to carrying out graded exposure are shown below. Write these down for the whole group to see. If possible, have parents write them down themselves to take away as a reminder. Discuss with the whole group how parents can help children to carry out exposure work at each of the steps. Exposure work does not always go smoothly, so it is helpful to discuss with the group strategies that can be used if there are hiccups in carrying out homework.

1. Construct a fear hierarchy
2. Choose a target and prepare for confronting the reminder - relaxation
3. Stay in the situation - positive self-statements
4. Monitor fear until acceptable level - fear thermometer
5. Self-praise and reward
6. Repeat with a more difficult target

Cognitive avoidance

Cognitive avoidance includes attempting to push traumatic memories away when they come to mind, and parents will probably have had direct personal experience of this, so again, make the link between adult and child experiences, and say clearly that children attempt to do the same thing. Cognitive avoidance also includes children not wanting to think or talk about the event. The value of talking and providing general emotional support may

have been covered already as in the sections above. In this group, children will have been shown in detail specific ways to deal with intrusive traumatic memories, which involve talking, as well as drawing and writing.

The general guidelines above that parents have already discussed apply just as much here. Parents need to know that their children will also practise more specific techniques aimed to help them process traumatic memories. Go through these writing and drawing techniques as in the children's groups. Explain the purpose of writing or telling a sequential story of a traumatic event as a means of processing intrusive memories, referring to the introduction to post traumatic stress. Parents can help by listening to the child as they tell the story of what happened, comforting when necessary and encouraging or asking for details if the child seems to get stuck. It is helpful to use the following structure:

- what was happening just before the traumatic event
- how they first knew something was wrong
- what happened next
- what the child did
- what the child saw, heard, felt, or smelled
- how the hurt happened
- how they knew it was over
- what they did afterwards
- what others did afterwards
- the very worst moment

It is important for parents to be able to see the child through telling the whole story, either with words or pictures. It is also important that the parent and child spend some time afterwards so that they allow themselves to recover from any emotional upset, perhaps using relaxation or distraction techniques;

or just playing together. The child should know that parents are still available to talk to afterwards.

At the end of this section going through the children's groups with parents, review the tasks that parents will be expected to do with children for homework. Ask if parents anticipate any problems in doing these. Ask if there are ways that parents can get together to support each other in helping their children.

5. Follow up

At the end of the group, it may be useful to plan some kind of follow up. Parents should be linked in with any psychological services for children at school. It is helpful to set up regular school-home links. Similarly, parents should know where to go if they continue to be worried about their children. Inform them of any specialist psychological services for children and families. As may have been mentioned already in the group, parents may want to set up self-help or support groups for themselves, or set up group activities for children in the community.

References

Dyregrov, A. (2008). Grief in children. A. Handbook for adults (2nd edition). London: Jessica Kingsley Publishers.

Dyregrov, A., Yule, W., Straume, M., & Kraus, F. (2011). Children and grief. Teaching Life Skills – A Children and War Foundation Manual. Children and War Foundation.

Krakow, B., & Zadra, A. (2006). Clinical management of chronic nightmares: imagery rehearsal therapy. Behavioral Sleep Medicine, 4, 45–70.

March, J.S., Amaya-Jackson, L., Murray, M.C. & Schulte, A. (1998). Cognitive behavioural psychotherapy for children and adolescents with posttraumatic stress disorder after a single incident stressor. Journal of the American Academy of Child and Adolescent Psychiatry, 37, 585–593.

Paunovic, N. (2003). Prolonged exposure counterconditioning as a treatment for chronic posttraumatic stress disorder. Anxiety Disorders, 17, 749–499.

Pennebaker, J. W. (1997). Opening Up: The Healing Power of Expressing Emotions, revised edition. New York: Guilford Press.

Shapiro, F. (1995). Eye Movement Desensitization and Reprocessing. Basic Principles, Protocols and Procedures. New York: The Guilford Press.

Spoormaker, V. I., & van den Bout, J., & Meijer, E. J. G. (2003). Lucid dreaming treatment for nightmares: a series of cases. Dreaming, 13, 181–186.

Wells, A., & Sembi, S. (2004). Metacognitive therapy for PTSD: a preliminary investigation of a new brief treatment. Journal of Behavior Therapy and Experimental Psychiatry, 35, 307–318.

Yule, W., Dyregrov, A., Raundalen, M., & Smith, P. (2013). Children and War: Past and present. The work of the Children and War Foundation. European Journal of Psychotraumatology, 4: 18424 - http://dx.doi.org/10.3402/ejpt.v4i0.18424

TRAINING GROUP LEADERS

The manual that accompanies these notes contains materials and detailed group protocols for child care professionals to run secondary prevention groups with children and parents who have survived war. The manual is intended to be sufficiently detailed for non-psychologists (e.g. teachers, social workers, community leaders) to run effective and comprehensive groups. Nevertheless, group leaders will usually require some preparatory training before running groups for children and parents.

This section therefore contains material and ideas for training group leaders. The aims of the session are:

- *to give group leaders a broader training in children's psychological reactions to war and intervention for children so that they will be able to adapt the group sessions as appropriate for their community*
- *to provide detailed instructions in how to use the accompanying manual*
- *to provide a model for group work that the leaders will use in their subsequent work with children and parents*
- *to set up a continuing support system for group leaders so that further group leaders are trained and a network of professionals is established*

These training sessions should be run as group sessions, with around 10 - 15 adults. Initially, visiting professionals will hold them. After initial training, it is expected that those local professionals who have experience of running successful groups with children in their community would go on to train their colleagues so that skills are spread in a cascade fashion. This section is therefore a guide both for visiting professionals and for group leaders who go on to be trainers.

Group leaders might be teachers, social workers, community leaders, nurses, youth workers, counsellors, psychologists, or other childcare professionals.

Have a copy of the group protocols for each of the members in this training group. They will need to refer to it throughout this training session, and then take it away to use when running their own groups. Also have copies of this section and the accompanying handouts and overheads ready to give out at the end of this training session, to enable group leaders to go on to train more of their colleagues in the future.

The style of these training sessions is as important as their content. Trainees will use their experience in this session as a model for their later group work. Therefore, use a combination of direct teaching, group discussion and brainstorming, small group work, role-plays, and direct practice of techniques to be taught. Sessions should be interactive and optimistic, promoting a sense of active coping without denying the seriousness of the difficulties. In particular, these training sessions are a way of mobilising the community, to also ensure that the training is relevant and at an appropriate level, and refer often to the practicalities of implementing groups in the community (availability of materials, venues, staff; co-operation of parents and schools; other ongoing programmes etc).

Emphasise that TRT trainings should be interactive, lots of activities and role plays. Role-play valuable for trainees' learning, and for the trainers to check that they have understood.

This applies to process skills in running the group (hence, opening and closing group role plays) as much as to delivery of tx components (hence, role plays on graded hierarchies eg).

Children's reactions to war

Find out the background, training and experience of the trainees in the group. Different backgrounds and training of group members will influence what and how you teach in this session. Ask if anyone has already attended training from other organisations, or if they are involved in other ongoing programmes. Find out what trainees expect of the teaching and be clear about your aims for this programme. At the end of this session, a concrete plan will be agreed with group leaders for implementation and follow up, but find out now if they anticipate major problems in following through on their training, otherwise this will return as an obstacle throughout today's teaching.

If you are an international trainer, be aware from the outset about differences between your own culture and the one you are teaching in. These will apply to some areas more than others (for example, the role of religion and ritual after death); and may apply to expectations about relations between parents or teachers and children; or more generally to the way children or childhood is viewed.

Depending on the level of training and experience of the group, you will need to educate about children's normal reactions to war, and put these in the context of posttraumatic stress for the purposes of introducing the three children's groups. From the outset, make links between adults and children's reactions. This will be helpful in the trainees' work with parents, and will also be useful to themselves, in terms of preventing burnout.

First, ask about what children have experienced. Ask for general descriptions of conflict in the area. Ask for examples of what children have witnessed or been involved in. For international trainers, it is especially important that adult trainees be given the opportunity early on to tell what has happened in their community. Without this, trainees may rightly feel that you cannot grasp the seriousness of what has happened. Giving this sort of abbreviated 'testimony' as to what has happened may also have beneficial effects for the participants. Even for local trainers, it is important to bear in mind that the severity and idiosyncrasies of conflict and violence can vary greatly from community to community, and you need to ensure that you and your trainees are aware of what has happened locally to cause stress reactions in children.

Characterise these events as traumatic. There may be a more appropriate local word. Throughout the children's manual, traumatic events are labelled in this way, but be sure to use a more child centred, locally appropriate word if one exists. Traumatic events are special because they are known to cause certain sorts of reactions in children and adults alike. Traumatic events during war are: sudden, intense, life threatening, cause injury, result in bereavement, involve intense sensory experiences - sights, smells, sounds - they are grotesque and horrific, and make you feel terrified and helpless at the time (see chart 1). How have children reacted to these sorts of events? Ask the group. Make a list (see chart 2). Get as broad a list as possible. Without either undermining the seriousness of these reactions, or labelling them as pathological, normalise them. That is, say that professionals working with children in a wide variety of countries at war have reported these sorts of reactions. The reactions are

common and expected; and there is something that can be done about them. The following may be helpful in summarising and explaining these reactions.

There is now good evidence from a variety of countries that have undergone civil unrest and violence or war in Europe, Africa, South America, and Asia, that children show adverse psychological reactions after exposure to the sorts of events above. Reactions may be broad, and include posttraumatic stress, depression, anxiety, grief reactions, and behaviour problems. These reactions depend foremost on what the child has experienced: those who have witnessed more atrocity are likely to suffer, and those who feel that their life has been directly threatened (i.e., thought that they were about to die) are particularly at risk. Girls tend to report more problems than boys do, but this may be due to girls reporting more, and attention should be paid equally to girls and boys. Over and above their direct exposure, the reaction of the child's family is the most important mediator. Children whose parents are showing strong adverse reactions are likely to suffer more themselves.

Some of the most common reactions of children are posttraumatic stress symptoms (see chart 3). PTS symptoms are by no means the sole consequence for children of having survived war, but a PTS framework is useful in understanding children's reactions and suggests ways to help. Show chart 3 (PTS symptoms in clusters) and compare it to the chart 2 or the list of problems the group has generated. Make the point that as adults we are fairly good at noting children's problem behaviour, but that it is less obvious that children have intrusive recall after exposure to traumatic events.

Children do experience intrusive recall of traumatic events. Use a case example of your own to illustrate (or see case example in first children's session). Ask the group for examples. Ask how intrusive memories show themselves (in response to reminders, out of the blue, in dreams and nightmares, and with younger children in repetitive play). This point may be well made by asking the group to consider their own reactions after witnessing atrocity. Adults may be well aware of intrusive images and nightmares, for example, from their own personal experience, and they need to know that children may react similarly.

It is important to explain why intrusive recall is important, and to link PTS theory to the real life problems noted by the group. Explain how intrusion and avoidance maintain each other in a vicious cycle (see chart 4). Use an appropriate local analogy here (bottling things up, letting off steam, slow release, etc) to explain how the cycle might be broken. Arousal symptoms often

accompany intrusive symptoms, and may maintain them. It is vital to link PTS to the problem list generated by the group. Encourage the group to think about how, if a child were suffering from PTS, it would help explain the changes in his behaviour (jumpiness, poor concentration, inattention, poor sleep, e.g.). Discuss what sorts of intervention this PTS framework suggests. At its core, it suggests that adequate processing of memories is needed if the vicious cycle between intrusion and avoidance is to be broken. Without going into details of intervention now, get the group to say this early on: at some stage, adults and children alike need to face up to what has happened to them. The PTS framework suggests a focus for help in each of the three areas: by working directly to change intrusive recall; by working to reduce cognitive and behavioural avoidance; and by working to reduce over-arousal.

As with the label 'traumatic events', this cluster of symptoms is referred to throughout the children's manual as 'Posttraumatic Stress Symptoms'. It is important in terms of normalising reactions that reactions be labelled somehow. However, there may be a more appropriate local or child centred term that can be used. Ask your trainees about this now. If there is a more appropriate local word, it should be used in place of PTS throughout this training and in the groups with children. Otherwise, it may be better for group leaders to ask their group of children to come up with a group name or 'nick name' for the cluster of symptoms, and to use this instead of PTS throughout the children's groups.

The three children's groups are structured around these three symptom clusters of PTS; and the parent group is aimed at supporting children in practising what they have learned at home. Before going through the detailed protocol for the groups, discuss with trainees the practicalities of running a group.

Running groups for children and parents

You will need to go over the following with the group. Ask if anyone has experience of group work with children, and ask them to say what it was like. What were the benefits? What were the problems (organisational or within the group)?

Why run groups?

We are now in a position to predict, fairly broadly, how children react psychologically to war. When whole communities are affected as in the case of war or political violence, many children may suffer. Although local and international organisations may set up treatment for children after (or during) the war, the burden of treating children may be reduced if we act early to prevent symptoms occurring in the first place. Groups are a way of reaching a large number of children as quickly as possible in order to teach self-help skills to prevent the need for later treatment. These groups are therefore community secondary prevention measures.

Who are they for?

After exposure to atrocities of war, everyone is at risk. These are not treatment groups for children who have been badly affected. Because they are prevention groups, it is intended that all children in a particular area would attend. The sessions are designed for groups of around 6 to 10, children aged about 8 years and older. Although these activities may be used with a fairly wide age range of children, the children in a particular group should be of roughly the same age or maturity level. Groups would normally be mixed, with boys and girls together in a group, unless there is some other reason to carry out single sex groups. The same group of children should continue through the three sessions together. Parents' permission for children to attend should be obtained, but all children should be strongly encouraged to attend. The activities demand a certain level of cognitive maturity, so these groups are not suitable for children with learning difficulties. Similarly, some of the activities here may not be suitable for children who are known to have been severely psychologically affected by the war, so consideration should be given as to whether individual treatment would be more appropriate for such children. Bereavement groups

might be run separately for children who have been bereaved; or it may be appropriate to include some of the material from the bereavement group in the three main children's groups. You should discuss this with your group of trainees.

Who should run them?

Groups can be run by a wide variety of professionals, such as teachers, youth workers, counsellors, nurses, pedagogues, community leaders. These are the sorts of professionals that today's group may go on to train later. Groups should only be run by people who have attended a training like today's. Normally, two colleagues would run groups, and it should be the same two people who run each session with a particular group of children so that continuity and trust can be built up. These same two co-workers should run the parent group for the parents of children they have worked with so that there is an established link between parents and children.

When should they be held?

Groups should be held as soon as it is safe to do so. In war, there are often curfews or bans on gatherings of groups of people, for safety or other reasons. If this is the case, it probably mean that there is a genuine threat to safety, and in addition to being risky, children would not benefit from the skills learned at a time of continuing threat. There needs to be a short period of relative calm before children will be able to absorb and remember what they will be taught in the groups. Discuss with the trainees the practicalities of when to start the groups, as this will depend entirely on the local situation. Once the groups have started, the parent group would normally be held first; and then the children's groups at weekly intervals for three weeks after that. This spacing of groups is to give children enough time to practise and consolidate skills from week to week.

Where should they be held?

No special equipment is needed for the groups, although it is helpful if group leaders have access to flipcharts or equivalent, pens and drawing paper and exercise books for children. Sweets and snacks or drinks might be in order. All

that is needed is a fairly comfortable room or safe place outside where the group will not be disturbed. Schools, or youth centres are often the best place.

What happens afterwards?

This needs thorough discussion with the group and should be returned to at the end of today's teaching. Ideally, today's trainees will go on to train their colleagues. This will establish a network of care providers. The network should work out ways to meet and support each other in the demanding work they are about to undertake. There may be other programmes run for children in the area, and the group leaders may be involved in these already, or if not, should link up with them. There will be a need to follow up activities learned in the groups. It is intended that this be done mainly at home with parents, but wherever possible, good links with schools should be made so that follow up activities can be done in schools. For some children, there will be a need for more specialist services, and the trainees should plan ways to refer children on where necessary to specialist agencies if available.

The style of the groups and therapeutic stance

The style of the groups is important and can be difficult to teach without direct experience. Be aware that trainees will use the style of today's training as a model for their groups later. These are not treatment groups, and the telling of very traumatic histories with considerable accompanying distress by individuals is not helpful. In other words, the groups should not become too heavy, or a source of secondary traumatization for other children. Rather, they should be active, optimistic, and positive. They should begin to counter feelings of being out of control and helpless; and promote self-efficacy and self-coping. The seriousness of children's problems should not be denied or belittled, but the stance taken is one of active coping, where the aim for children is to begin beating back their symptoms to regain control over how they feel and what they do. Return to this theme in today's session, both in the way you are teaching, and explicitly.

Say that at the end of the teaching, a concrete plan and timetable of children's groups and professional support groups will be agreed.

The children's groups

The aim of the training is to empower the trainees to use and adapt different components or modules in the manual. It is not intended that the manual be used as a very strict protocol. The basic principles need to be adhered to, but differences in children's experiences or problems, or trainees' background and experience means that a flexible approach is called for. Where possible, these techniques should build on or be included as part of local helping techniques. Trainees should feel sufficiently confident at the end of the training to use and adapt these skills in the most appropriate way.

Practicalities

Over and above the style or therapeutic stance of the children's groups, you will need to discuss with your group of trainees the practicalities of running a group for children. Depending on their level of experience of working with children or running groups, you might want to raise issues such as seating arrangements, how often to take breaks, which ground rules to set, what to do if someone dominates the group, how to react if someone becomes upset, and so on. Discuss these issues now so that any anxieties the trainees have about working with groups of children can be dealt with early on so that they do not interfere with the rest of the teaching.

In the discussion, try to bring out the following. It is usually best to have the children seated informally in a circle, on the floor, cushions, or chairs. The group shouldn't resemble a classroom teaching session. Breaks should be taken often and when needed. Find a balance between getting through the material, and making sure children are alert and receptive. A list of ground-rules is contained in the first children's group notes. It is important that these are set early on, so that the group knows what is expected of them, and so that they can be returned to as and when necessary. Encourage the children to generate these rules themselves, not have them imposed by the facilitators. Group leaders should expect that some children would be more vocal than others. They should try to encourage quieter members to participate, but remember that they can only do this at their own pace. Praise effort rather than achievement. If someone is dominating the group, thank them for contributing, but remind them that everyone should have the chance to speak. Discuss with your group of trainees what is the best way to handle strong emotions in the

group. If a child becomes upset, the leaders should judge whether to carry on or not. In some cases, the group might be able to help the child - for example by waiting until he or she has recovered, by saying that they too get upset and how they deal with it, that it's OK to cry, or by offering comfort. In other cases, it may be necessary for one of the facilitators to leave the group with that child for a short time. The facilitators need to find a balance between accepting strong emotions, and not letting the group be dominated by them, becoming too 'heavy'. This requires some therapeutic skill, and should be discussed fully with the trainees now.

Structure and principles of the groups

Each of the three children's groups is structured in the same way, beginning with group warm up exercises, going on to direct practice of skills, and closing with homework assignments. Discuss why warm ups are useful, and maybe practise one now with this group of trainees. Discuss the importance of homework assignments, emphasising again that these are preventative skills teaching groups. The analogy used is one of building up a toolbox of tricks to beat back the normal but distressing reactions; there may be a more appropriate local metaphor.

The principles behind each group are outlined at the beginning of the notes for each session. They are based on the model of stress reactions above. The first session aims to help children regain control over intrusive recall as a first step in beating back their PTS. The second session teaches relaxation skills, useful in itself to combat over arousal, but also as a preparation to carrying out exposure work. The last session teaches detailed methods for graded imaginal and in vivo exposure, aimed at reducing avoidance.

Normalising and labelling reactions

Children experiencing symptoms of anxiety and posttraumatic stress commonly feel out of control. They fear their own reactions, feel that they cannot do anything about them, and fear that they may be going crazy. It is therefore helpful for them to know that these reactions are common (normalising) and have a well-recognised name (labelling). Normalising and labelling is a first step to begin to make sense of the chaotic symptoms so they become more manageable. On the other hand, normalising is not meant to belittle children's

reactions; and labelling is not meant to pathologize them. To avoid this, rather than using clinical sounding terms such as Posttraumatic Stress or Traumatic events, trainees should find an equivalent local phrase to use with children. In the children's groups, one of the first tasks would be for the group to come up with a group- name or nickname for posttraumatic stress. This has the equivalent effect of labelling and making reactions seem more manageable, without using unnecessarily complicated or clinical terms. Have the group of trainees come up with some suggestions as to what children may call PTS and traumatic events. Throughout the group protocols, the terms PTS and traumatic events are used, but group leaders should substitute the children's chosen term for this.

Training Day 1: Intrusion groups

The aim of the intrusion group is for children to regain control over intrusive recall. It comprises the following activities: education about stress reactions; safe place techniques; image, sound, and olfactory changing techniques; dual attention tasks; dream restructuring; and distraction. In this training session, you should teach these skills to the participants so that they have direct experience of them; and train them in how to teach these skills to children.

*Following introductions, start by inviting participants to talk to each other in pairs or threes about their **expectations and wishes for the workshop**, and then to feedback. This is helpful in setting the scene for the three days. It also allows the group leader to be clear about the scope of the TRT groups.*

*The group is then asked to **describe the effects of trauma exposure on children**. This is best done as a whole group exercise, and one of the group leaders will write examples of traumatic stress reactions on the flip board, if available. This is an opportunity to emphasise the difference between memories and intrusions (including the here and now quality of intrusive memories).*

*After having described the manual content, and the beginning of the first group, invite participants to split into two or three groups of 8 to 10 participants each (depending on the size of the group). In each subgroup, two participants are to **role-play group leaders**. Their task is to introduce the group, and establish the group rules. In a second role-play, in the same subgroups, two*

different participants are invited to role-play group leaders eliciting and normalising trauma reactions and reminders.

*In the next exercise, the group leader leads a **safe place exercise**, taking care to – avoid memories of places associated with loss.*

***Imagery techniques** are described and demonstrated by the group leader, and then participants separate into pairs to practice giving instructions to each other on how to implement imagery techniques.*

*For the **dual attention tasks**, it is best that the group leader leads the whole group in tapping. Remember that the content of traumatic memories is not explored when using dual attention tasks, and there is therefore no need to follow through or interpret memories. If there is time, it is helpful if one or two volunteers then lead the whole group in tapping.*

*For the **dream restructuring** techniques, when leading a whole group discussion, it is especially important to be aware of any differences between your culture and the one you are training in. Are there any specific beliefs about dreams in this community? Do dreams have any special meaning? Do your trainees find symbols in dreams? Whatever local beliefs about dreams are, in addition, we know that they are manifestations of intrusive posttraumatic memories, and that they are amenable to change. Say this directly, and say that rehearsal relief and dream restructuring are techniques that are known to work with adults and children alike.*

*For the **distraction tasks**, have the trainees first list these for themselves (this will be useful later on when teaching about burnout prevention) so that the value of distraction is emphasised; then think briefly about distraction activities for children.*

*In teaching about closing the group, stress the importance of each child contributing something, and of the group leaders praising something specific about each child. The children's group should finish on a positive note, with each child ready to practise the homework tasks and to return the following week. If there is time, it is extremely helpful for participants to finish this day with a **role-play** in which they have the opportunity to practice closing the group and giving homework. This can either be done in the same subgroups is at the beginning of the day, or as a plenary to the whole group.*

Training day 2: Arousal group; avoidance group 1

The main aim of the arousal group is to teach children relaxation techniques.

*After summarising day one briefly, and **inviting questions and clarifications**, we move swiftly to a **role-play**, in which two participants role-play group leaders while the remainder role-play children, and the task is to open the group and review homework. Participants role-play children are encouraged to be inattentive and/or distressed to ensure that the role-play is as true to real life as possible. This role-play is best done in smaller subgroups, as in the previous day.*

*Next, in a whole group, ask participants for a **list of physical sensations** that accompany fear and anxiety. As always, use an interactive rather than didactic style of training, remembering that trainees will model their practice on yours.*

*Demonstrate how to use a **fear thermometer**, and invite local examples of how the subjective intensity of fear might be measured.*

Next, ask participants what methods they use to relax (and ask them to demonstrate) , and what ways children use to relax.

*Teach deep **muscle relaxation and breath control** relaxation as in the manual. If you have a chart of major muscle groups, it can help with this. Remind the group that children too may become self-conscious or embarrassed doing this in a group; and discuss ways to handle this.*

If there is time, have a member of the group lead the rest of the group through the deep muscle relaxation. They may want to combine this with other safe place techniques as above, or with local practices.

*We then use brainstorming techniques to generate helpful benign neutral **coping thoughts**.*

*For the **activity scheduling**, the main point is that goals are realistic and achievable; and steadily built up from week to week. Activities must be safe and achievable. Show an example of a diary (see chart 4) and run through how to use this with trainees.*

Next, when training participants how to run group 4 (countering behavioural avoidance), it is helpful to have the list of reminders that were generated on day one to hand. Invite participants to add new reminders, if they would like to. Following this, each participant is invited to spend a few minutes making a ***list of individual reminders*** *(not graded), role-playing a child if that is easier.*

In the large group, give a ***rationale for graded exposure*** *using a non-trauma related fear or phobia such as height phobia. Demonstrate to the large group how to grade reminders of exposure.*

Next, in pairs or threes, participants are asked to ***role-play*** *child and a group leader. The child selects one of the reminders from the list they have just generated, and the group leader supports them in breaking this down into a graded hierarchy.*

Following this, one or two pairs may ***demonstrate the process of designing a graded exposure plan*** *to the whole group.*

To complete this group, it is helpful if the group leader then role-plays to the whole group given clear ***instructions for carrying out graded exposure as homework****.*

If there is time, one or two of the participants may ***role play closing the group*** *and setting homework*

Training day 3: avoidance group 2, groups for parents, implementation

After summarising day two briefly, and ***inviting questions and clarifications****, we move to a* ***role-play****, in which two new participants have the opportunity to open the group and review homework.*

Next, as a whole group, we begin the session by inviting ***discussion*** *about the advantages and disadvantages of cognitive avoidance (talking and thinking).*

The group leader then models supporting children in talking writing and drawing about their traumatic experiences. We emphasise the importance of writing or drawing the event as a sequential story.

At the end of this section, we encourage out role play in which participants close this group, and discuss with children and their hopes for the future, ensuring that the groups are ending on a positive note.

The Parents' Group

The parents' group is an important component of intervention for children because: 1. Parents can help children practise homework tasks; and 2. Parents can use self-help strategies to look after their own needs so that they are in a better position to help children.

Find out from your group what their attitudes to parents are. Different professional groups may have different attitudes to the role of parents in general. You should try to counter any feelings of blaming parents on the part of your group. Find out what role parents as a group have played so far in helping schools, children, or in rebuilding programmes in general. Find out for example if there are any established school-home links if you are working with teachers.

The parents' groups have a number of aims: to educate parents, to help them assist their children, to help them look after themselves, and to 'mobilise' parents to act as a community on behalf of children. Ask the group how they would do this. Having run children's groups, trainees will be in a good position to adapt the parents groups, perhaps more than the children's groups. Sometimes, it will be necessary to stress to trainees that parents are their allies in helping children, and not to lecture or talk down to parents.

Depending on what the group comes up with as a plan for parents groups, run through the group notes with them. As with children and the current group, parents will benefit from information that will help them understand their children's reactions. This is probably best taught to parents in the same interactive way as with trainees, and guidelines for doing this are contained in the parents' group notes.

The group of trainees may decide that parents will need convincing of the need for an emotionally supportive environment at home where children feel able to express themselves; and practical ways to do this. Ideas for doing this are contained in the parents' group notes, and you may want to go through some

of these exercises with your group now. If necessary, reframe this as experiential practice for the group.

For example, discuss with the group why children may not want to talk. These might include:

- *it is too painful*
- *they lack the words or concepts to express what they feel inside*
- *feelings and emotions are contradictory*
- *they feel guilt and confusion*
- *they are afraid of not being heard*
- *they are afraid of losing control*
- *they need time to build up trust*
- *they do not feel the need to talk, or feel that they have talked enough already*
- *they have others they talk to*

Discuss the benefits of talking (see parents group notes). These might include:

- *gets things out in the open*
- *child can understand better*
- *relieves tension for child*
- *makes the child feel better*
- *talking makes it easier to forget*
- *child can start to distance self from painful memories*
- *puts their fear in perspective*
- *means they don't bottle everything up*
- *we can help them if we listen to their problems*
- *child can get a different perspective on things*
- *child can hear his feelings are normal and he's not going crazy*

Discuss what is needed for children to talk. Mainly, this will be a trusting relationship where the child feels he will be heard and taken seriously; where the adult can hear his pain and be 'containing' and offer comfort and practical advice; and a feeling on the part of the child that he will not lose control and scare the adult away.

Stress the importance of parents taking care of their own emotional needs. Stress also the importance of parents understanding the skills children were taught and helping them to practise them.

Encourage your group to think of ways that they could provide continuing support for parents to help their children (and return to this later at the end when drawing up a concrete plan and timetable).

Group leaders: Burnout Prevention

Before finishing this training, trainees need to be aware that they too must look after their own needs. They will have had experience of the self-help techniques during the training, and should be encouraged to make use of these themselves.

In particular, they need to be aware of the potential for professional burnout. Either as a whole group discussion to finish the training, or in small groups, ask the trainees what difficulties may arise if they push themselves too hard. They might make a list that includes:

- *lose interest in work*
- *become depressed and demoralised*
- *get low in energy*
- *feel that there's no point in anything*
- *get bored*
- *carry on in robotic way, often working longer and longer*
- *don't give self time to think, relax*

- *develop physical illness (aches and pains, nausea, tiredness, headaches)*
- *have sleep problems*
- *substance use (drinking and smoking more than usual)*

The consequences of this are severe. The person is no longer available as an emotional resource for anyone, their work declines, and their family and other relationships suffer. The trainees owe it to themselves and their community to look after themselves. How can they do this? Again, this is often best done as a lively group session. The group might come up with:

- *a change in attitude*
    - *recognise that they can't do everything*
    - *recognise that they're not indispensable*
    - *recognise that they're not responsible for everything*
- *share or vary their workload*
- *take holiday, schedule time for rest and relaxation*
- *confide in someone, find someone to talk to*
- *give self permission to enjoy things*
- *plan time with family, friends - and stick to it*
- *eat well*
- *exercise*
- *organise proper support and supervision at work*

The group may want to organise professional support or supervision networks below.

Implementation

If this training is to be followed though effectively, it is our experience that a concrete plan of action needs to be made now.

The plan will depend entirely on the local situation: the numbers of schools and children to be reached, the number of local trainers and group leaders available, other ongoing programmes for children, the availability of resources or incentives, and the current security situation.

The group should plan now how to implement the groups in as detailed a way as possible. This might include how to start the groups, how to inform relevant people (authorities, schools, parents etc), who is to work as pairs together, and which schools or groups they will cover. A timetable should be agreed and written down. People to act as trainers in the future should be nominated now, and a plan timetabled for further training.

How is the group going to monitor and support their work? Is it possible to set up regular supervision or support groups for leaders? The group may want to extend the programme, for example by running further parent support groups, or by extending activities into schools. This may be done with other ongoing programmes for children.

Evaluation should also be discussed and planned now.

APPENDIX I: Alternative relaxation exercises

This section contains: -

Jacobson's Deep muscle relaxation for older children

Deep muscle relaxation for younger children

TOOL: Progressive Muscle Relaxation (older children)

Note

Relaxation training can sometimes make children feel embarrassed and uncomfortable. This means that you need to teach the skills first in a straightforward and direct way. Keep an eye out for any children who seem particularly uncomfortable or jittery. One of the co-leaders can quietly help them when the group splits up for practice.

The group sits in a wider circle than usual. It is easier if they are sitting on chairs rather than the floor. Make sure everyone is in a comfortable position, with enough space around them (at least double arm width apart). One co-leader should teach and demonstrate these skills; the other can model learning and practising them with the group. As you go through these instructions, demonstrate them so the group can see. Shorter, simpler instructions for younger children are given below (Tool: Body Controller). It is helpful with children of all ages to quickly run through naming of different body parts or muscles beforehand.

We'll try out the relaxation in a minute. Here's what to do: this is just for practice, so watch me, and copy what I do. We're going to focus on different muscles in our body in turn, first of all making them tense and hard, and then releasing them and relaxing them, OK? I'll talk you through the different muscles now, one by one.

Make sure you're comfortable. Just breathe nice and steadily.

First, your hands and arms. Make a fist with both hands. Clench the fist really hard. As hard as you can. Feel how tense it is. Feel the tension in your hands and forearms. Hold it [Tension for all the different muscle groups should be maintained for roughly a slow count of five]. Now relax. Let it go. Good. Now your biceps. Hold your arms like this [Arms to the side, palms upwards, arm bent at the elbow, as if you're lifting a heavy weight]. Make them really tense. Feel the tension in your biceps muscles. Feel how hard it is. Now relax, let it go Good. Next your triceps. Hold your arms out straight like this [Arms locked straight out in front]. Make them really tense. See what the tension feels like at the back of your upper arms. Hold it. Feel the tension. And relax. Good.

Now your neck and shoulders. Shrug to shoulders up to your ears like this. Feel the tension around your shoulders and neck. Hold it, feel how tense it is. And relax. Now let your head drop forward so your chin rests on your chest, like this [this can't be done if children are lying down]. Roll your head round to the side like this. Roll slowly back to the front. And to the other side. Now rest your chin on your chest again. Good. Bring your head up.

Next your stomach. Make your stomach really hard and tense. Hold it. Feel the tension. And relax.

Next your legs [this is difficult to do unless children are sitting on chairs or lying down]. Hold your legs out in front of you like this [legs straight out, locked at the knee], and really tense them up as much as you can. Hold them tense. Feel it really hard. Hold it. And relax. Let it go. Good.

Last your feet. With your legs in front of you, point your feet away. Push it hard. Feel the tension in your feet and calf muscles. Hold it. Feel the tension. And relax. Let it go.

Note

You've now been through the major muscle groups. Do not expect children to be relaxed now - this was practice at isolating and tensing the different muscle groups in turn. Ask how it went. Depending how the practice has gone, you may want to run through this once more, with the different muscle groups in turn: hands and arms, neck and shoulders, stomach, legs, feet.

After a quick break (children stand up, move around, shake themselves), have the group split up and sit or lie around the room individually. Go through the muscle relaxation again. This time, develop your pattern. Speak slowly, in a calm and steady tone of voice. Emphasise the relaxation in contrast to the tension. Make suggestions that they feel relaxed. Suggest that their bodies feel heavy and warm, for example. Remember the relaxation you carried out in training. Develop your own "script" to guide the children through, one that you are comfortable with. Here is a suggestion that you can develop as you wish….

Make sure you're comfortable. Just breathe calmly and deeply and steadily. When you're ready, let your eyes close. Notice your breathing, steady and slow. With each breath out, you begin to feel a little more relaxed. Steady breathing, feeling calm and relaxed.

Now we'll go through your muscle groups tensing and relaxing. First, your hands and arms. Make a tight fist with both hands. Clench the fist really hard. As hard as you can. Feel it. Feel how tense it is. Feel the tension in your hands and forearms. Hold it.... and relax. Let it go. Notice how the tension leaves your arms.

Now your biceps. Hold your arms bent at the elbow as before. Make them really tense. Feel the tension in your biceps muscles. Feel how hard it is. Hold it.... and relax. Let it go. Notice the difference between the tension and relaxation.

Next your triceps. Hold your arms out straight. Make them really tense. Notice the tension at the back of your arms. Hold it. Feel the tension. Feel it. Hold... and relax.
Now your arms are relaxed. They begin to feel heavy by your sides. Notice how the tension has left your arms. Steady breathing. You're feeling calm and relaxed. With each breath out, a little more tension leaves your body.

Now your neck and shoulders. Shrug your shoulders up to your ears. Feel the tension around your shoulders and neck. Hold it, feel how tense it is. Hold it.... and relax. Now let your head drop forward so your chin rests on your chest. Roll your head round to one side. And slowly back to the front. And slowly to the other side. And roll it back to rest your chin on your chest again. Bring

your head up. Notice how the tension has left your neck and shoulders. You're feeling calm and relaxed.

Now your stomach. Make your stomach really hard and tense. Hold it. Really hard. See what the tension feels like. Feel it. Hold it... and relax. See the tension leave your stomach.

Your stomach... and shoulders... and arms feel relaxed. The tension is leaving your body. You're feeling calm, and relaxed, warm. With each breath out, you feel a little more relaxed. Your body feels heavy against the floor. Steady breathing. And relax.

Now your legs. Hold your legs out in front of you. Make them really tense. Feel the tension in your legs. Notice what it feels like. Feel it really hard. Hold it... and relax. All the tension is leaving your legs. Steady breathing, and you're feeling heavier, and calm... and relaxed, and secure.

Now your feet. Point your feet away from your legs. Push them hard. Feel the tension in your feet and calf muscles. Feel it harder. Hold it. Feel the tension. Hold it... and relax. Let it go. Notice how the tension is leaving your legs and feet and you feel calm and relaxed and secure.

Now your body is relaxed. Steady breathing... With each breath out you feel more relaxed. The tension is leaving your body. You feel heavy, and relaxed, and calm, and warm. Just steady breathing. Good. Heavy, warm, relaxed. Think about your arms… and the tension leaves them. And your neck… feels relaxed. Notice your stomach, and the tension is just leaving your stomach. Legs… feel heavy and relaxed. And feet… relaxed.

You feel calm .and steady… and relaxed. Just stay like this for a minute. With each breath out, say the word relax to yourself. Feeling calm and relaxed. And when you're ready, let your eyes open slowly. Just sit. Notice how you feel.Relaxed.Good.

TOOL: Body Controller (younger children)

Note

Teaching Relaxation by noticing the difference between tension in different muscles and relaxation. The following instructions are based on adult progressive deep muscle relaxation. Use a calm, soothing voice and a regular rhythm. You may notice the children begin to breathe in time with your phrasing. You can use this to either increase the depth of their relaxation or make them more alert towards the end by altering the speed at which you present.

We'll start by showing you a trick to relax all those bits you spotted on [name of drawing]…your hands, your arms, your legs…all the bits. And we're going to do them one by one. We'll get you to make them all tight and then make them relaxed and floppy. We'll do it too, so if you don't know what to do just have a look at us. OK make yourself comfortable. Spread out a little, so you've got enough space to lie down/stick your legs out in front of you. Good.

> →***Do*** *the exercises as you read out the instructions so that the children can look at you to see what to do if they are unsure.*

Now just breathe nice and evenly. In… and out… in…and out…. Make sure you're comfortable. When you're ready you can let your eyes close, if you want. Whatever's best. Just make sure you're comfortable. Notice your breathing. With each breath out you begin to feel more calm… more relaxed. Just breathe calmly and evenly. In…and out…in and… out. Now we'll go through each bit of your body in turn making it feel all hard and uncomfortable. Then I'll ask you to relax it and make it feel all floppy and floaty and relaxed.

Fingers and hands: squeezing a lemon

We'll start with your fingers and hands. Make a tight fist with both hands – like you're squeezing a lemon. Really tight so you can feel the juice squishing out your fingers. Feel it. Feel how tense it is. Feel the tension in your fingers, in your hands and the bottoms of your arms. Now RELAX and let it go… let your fingers gently open up like a flower. All relaxed and comfy. And notice how different it feels. Now let your fingers and hands stay all relaxed and we'll move on to your arms.

Lower arms: lifting a weight

Now, bend your arms at the elbow like you're lifting a really heavy weight. Feel how hard and uncomfortable your lower arms feel. All tense and stiff. Now RELAX…and let them fall down again all nice and heavy and relaxed. That's better. All floppy and relaxed.

Upper arms: wooden sticks

Now for the tops of your arms stick both arms out straight in front of you, all stiff and hard like they are made out of wood. Feel how hard they are… all tense and uncomfortable. Now let them go all floppy and loose, as they flop down all heavy and relaxed. Nice and relaxed. And notice how your arms and your hands and your fingers all feel heavy and floppy and relaxed. As you feel all floaty and relaxed.

Head and face: facial expressions

Now for your head and face…. Lift up eyebrows like you're really surprised. Try and make them touch your hair…. Now squeeze them tight and frown. Notice how your head and your face and your scalp…and all the way round the tops of your ears feel all hard and uncomfortable. Now relax…just let your face go all floppy and relaxed, no smiles, no frowns… no expression at all like you're just about to fall off to sleep, with your mouth all floppy and relaxed.…

Let the relaxation flow all over your face and your head and your scalp, down your arms and hands to the tips of your fingers. All floppy and relaxed

Shoulders and neck: turtle or tortoise in its shell

Now for your neck and shoulders. Just pretend you're a tortoise or a turtle and bring your shoulders up to your ears like you're covering your head with your shell. Shrug your shoulders up to your ears. Try and cover your head. Feel how hard and uncomfy it feels around your shoulders and neck. Hold it... and relax. Bring down that shell. Feel how comfy your shoulders and neck feel. Now let your head gently flop forward like you can't even hold it up. Let your chin rests on your front and gently roll your head round to one side...and then slowly back to the other side. Now roll it back to rest your front again. Now bring your head back to normal and notice how your neck and shoulders feel all relaxed and comfortable. You're feeling calm and relaxed.

Tummy: breathing

Next your tummy….Now breath calmly and evenly. Make your tummy go up and down when you breathe… calmly and evenly… in… and…out…in…and...out. Now

push your tummy out and make it all hard and uncomfortable.... See how it's hard to breathe when your tummy's all hard and uncomfy. Now RELAX…and let yourself breathe calmly and evenly in… and… out. In… and out. And notice how the relaxation increases on each breath you make out.. Calm and Relaxed. All floppy and comfortable. Heavy and relaxed….Breathing calmly and evenly.

Legs and feet: pushing down and turning up toes

Now your legs.[this is difficult to do unless children are sitting on chairs or lying down]. Hold your legs out in front of you like this [legs straight out, locked at the knee]. Make them feel all stiff… and relax. And your feet. Point your feet away from your legs. Push them hard. Feel the tension in your feet and the backs of your legs…and relax. Let it go. Notice how your legs and feet feel all heavy and relaxed.

Whole body: heavy, floaty, calm, relaxed

Now let your whole body feel all floppy and relaxed, all heavy and comfortable as you breathe calmly and evenly. With each breath out you feel more relaxed more floaty…as your tummy goes up and down… all comfy and relaxed…and your shoulders feel comfy and relaxed…and your face feels all floppy and relaxed…as the relaxation floats all the way down your arms and your hands to the tips of your fingers. Heavy, floaty, calm and relaxed.

Just stay like this for a little bit and with each breath out, say the word "relax" to yourself. Feeling calm and relaxed. And when you're ready, let your eyes open slowly. Just sit. Notice how you feel. Relaxed. Good.